



**CONNECTICUT DEPARTMENT OF PUBLIC HEALTH
COMMUNITY, FAMILY HEALTH AND PREVENTION BRANCH
MATERNAL, CHILD HEALTH, AND ACCESS TO CARE SECTION**

**State Plan of Program Operations
Special Supplemental Nutrition Program
For Women, Infants, and Children (WIC)**



**Federal Fiscal Year 2025
(October 1, 2024 – September 30, 2025)**

**Submitted in accordance with USDA
Food and Nutrition Service
Federal Regulations 246.4(A) - State Plan**

August 15, 2024

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A. INTRODUCTION

The Special Supplemental Nutrition Program for Women, Infants and Children (WIC) is implemented by the United States Department of Agriculture (USDA-FNS) under public law 95-627, Section 17 of the Child Nutrition Act of 1966; final regulations were issued January 2002. Funds for Food and Administrative costs are transferred from USDA–FNS to the Connecticut Department of Public Health, Community, Family Health, and Prevention Branch, Maternal Child Health and Access to Care Section).

The State Plan for Connecticut’s WIC Program is the governing document that provides guidance and direction for the State agency and local agencies administering the program. In compliance with Federal regulations and State regulations and requirements, the plan is updated annually to ensure the inclusion of new and revised federal and state requirements and annual accomplishments and new goals and objectives. Although the State Plan is primarily based on Federal regulations, including requirements and guiding principles for best practices from the state perspective and that of the nation’s public health framework.

The WIC State Plan references a single document but has three major components.

Section I of the plan contains the State goals and objectives FFY2025 and the evaluation for FFY2024. To the extent possible, the goals address the core functional areas of the WIC Program. The functional areas are management and organization, nutrition services and breastfeeding support and promotion, food delivery and food instrument accountability, vendor/retailer management, management information systems, caseload management and outreach, coordination of services, civil rights, certification and eligibility, monitoring and QA, fiscal management and data quality, analysis, and reporting. The goals and objectives are State-specific and function as a guide for enhancing both State and local program operations effectiveness and efficiency. The WIC Vendor Agreement is included for review. The Connecticut WIC Program’s FFY2025 goals and objectives include upcoming program changes, such as the transition to a new MIS maintenance and operations contract and the conversion to a new EBT provider, but also include activities focused on preparing for required federal changes in the Food Package Final Rule, planning and implementing modernization efforts to improve the participant experience, and improving disaster plans and continuity of operations efforts as per the Access to Baby Formula Act (ABFA). The draft State Agency Internal Operating Procedures Plan is included as an attachment. Further, State Outcome objective reports continue to undergo a QI process which have impacted State agency targets for the FFY2025 State Plan. Close to completion, these changes to reporting will improve program outcomes tracking and evaluation in the future and allow the Connecticut WIC Program to better monitor and respond to trends.

Section II is the local agency operations manual and provides guidance to State and local staff about clinic level WIC policies and procedures. In addition to the State Agency Internal Operating Procedures Plan, included with this submission are updates to the previously approved Disaster policy for USDA review and approval.

Section III outlines the State level operations as Functional Format Checklists. See Appendix B for the relevant updates.

Approximately \$52 million is allocated to Connecticut WIC for Food and Nutrition Services Administration funding and an additional \$12 million rebated by **Abbott Laboratories (in the process**

of being extended through September 30, 2026, via NEATO), through a cost savings measure as part of the infant formula rebate program.

B. MISSION STATEMENTS

DPH Mission:

To protect and improve the health and safety of the people of Connecticut by:

- Assuring the conditions in which people can be healthy.
- Promoting physical and mental health, and
- Preventing disease, injury, and disability.

DPH Vision Statement:

Healthy People in Healthy Connecticut Communities

Community, Family Health, and Prevention Branch Mission:

The Connecticut WIC Program is in the Maternal, Child Health, and Access to Care Section, which is one of two sections in the Community, Family Health and Prevention Branch at the Connecticut Department of Public Health. The CFHPB is a positive and productive branch that creates and achieves optimal public health outcomes through strong, consistent, proactive and ethical leadership; a positive and productive workplace environment; results-based accountability; and, premier customer service to the public by valued employees through technical assistance, best-practice and research-based expertise, and clear and accurate communication.

WIC Program Mission:

The Connecticut WIC Program is committed to improving the health of eligible pregnant women, new mothers, and children by providing nutrition education, breastfeeding support, healthy foods, and referrals to health and social programs during the critical stages of fetal and early childhood development.

Breastfeeding Statement:

The Connecticut WIC Program endorses the American Academy of Pediatrics' Policy Statement on "Breastfeeding and the Use of Human Milk" (2022), which states, "Breastfeeding and human milk are the normative standards for infant feeding and nutrition. The short- and long-term medical and neurodevelopmental advantages of breastfeeding make breastfeeding, or the provision of human milk, a public health imperative. The American Academy of Pediatrics (AAP) recommends exclusive breastfeeding for approximately 6 months after birth. Furthermore, the AAP supports continued breastfeeding, along with appropriate complementary foods introduced at about 6 months, as long as mutually desired by mother and child for 2 years or beyond."

The Connecticut WIC Program promotes exclusive breastfeeding as the normal infant feeding method through the first year of life and beyond, with the addition of appropriate complementary foods when the infant is developmentally ready, usually around six months of age. All WIC staff have a role in promoting and providing support for the successful initiation and continuation of breastfeeding.

WIC Program Vision:

A WIC participant is the most important person to enter the WIC office. A participant is not dependent on us. We are dependent on them. They are the purpose for our work. We are not doing a favor by serving the participant. It is the participant who is doing us a favor by letting us serve them. A participant is a person who brings us their wants. It is our job to handle their needs, with professionalism and efficiency, always with each participant in mind. –*Adapted from Hot Pots restaurant menu, Customer Service statement*

C. STATE AGENCY ORGANIZATION

STAFF MEMBER	PRIMARY RESPONSIBILITIES
Marcia Pessolano, MPH, RDN, CD-N State WIC Director T: (860) 509-8101 F: (860) 509-8391 E-mail: marcia.pessolano@ct.gov	Federal grants management Contracts and budgets WIC program policy Program planning and evaluation Program management & administration Information Technology & Fiscal Units' oversight Certification and Eligibility
Amanda L. Moore, MPH, CLC State Nutrition Coordinator T: (860) 509-8055 F: (860) 509-8391 E-mail: amanda.moore@ct.gov	Supervisor of Nutrition Services Certification and Eligibility Local agency monitoring and evaluation review lead Local staff training, liaison & technical assistance State Plan Management and policies Grant management activities Local agency RFP development Outreach MIS/EBT
Marilyn Lonczak, MEd, RD, CLC Nutrition Consultant 2/ State Breastfeeding Coordinator T: (860) 509-8261 F: (860) 509-8391 E-mail: marilyn.lonczak@ct.gov	Breastfeeding promotion and support, planning and evaluation Breastfeeding Peer Counseling (back-up) State Plan Management and policies Local staff training, liaison & technical assistance Nutrition Risk Criteria (back-up) on RISC WIC & Head Start Better Together Collaboration (sustainability) Operational Adjustment (OA) proposal coordination Representative for WIC-DPH on CBC, Food Policy Council, SEPI-CT, and other interagency workgroups
Pamela Beaulieu, CLC Nutrition Consultant 2 T: (860) 509-7138 F: (860) 509-8391 E-mail: Pamela.Beaulieu@ct.gov	MIS/EBT and CT-WIC coordination CT-WIC HelpDesk (scheduling and clinic modules) CT-WIC Nutrition Unit Reporting liaison Local staff training, liaison & technical assistance Update/provide input on Local Agency Plans/State Plan Outreach WIC Shopper
Amy Botello, RD, CLC Nutrition Consultant 2 T: (860) 509-7656 F: (860) 509-8391 E-mail: amy.botello@ct.gov	Nutrition Services & Certification Reviews Issue written reports/respond to corrective action plans Provide technical assistance Nutrition Assistant II training coordination Update/provide input on Local Agency plans/State Plan Outreach Digital Nutrition education materials project lead

	WIC Shopper and WIC Smart coordination One Call Now (OCN) and social media
Luz Hago, RD Nutrition Consultant 2 T: (860) 509-7662 F: (860) 509-8391 E-mail: luz.hago@ct.gov	WIC contract formula and eligible nutritional's lead Formula approval and issuance Develop new food packages Local agency liaison, technical assistance Nutrition education Update/provide input on Local Agency Plans/State Plan
Mellessa McPherson-Milling, CLC Nutrition Consultant 2 T: (860) 509-7814 F: (860) 509-8391 E-mail: mellessa.mcPherson-milling@ct.gov	Local agency Program Operations Reviews Issue written reports/respond to corrective action plans Civil Rights Update/provide input on Local Agency Plans/State Plan
Lori Goeschel MS, RD, IBCLC Nutrition Consultant 2/ State Breastfeeding Peer Counseling Coordinator T: (860) 509-7755 F: (860) 509-8391 E-mail: lori.goeschel@ct.gov	Breastfeeding Peer Counseling Breastfeeding Peer Counseling contract management, program planning, technical assistance, and evaluation CDC 1807 cooperative agreement breastfeeding activities (back- up)
Kimberly Boulette Health Program Supervisor T: (860) 509-7845 F: (860) 509-8391 E-mail: kimberly.boulette@ct.gov	Supervisor Food Resource & Vendor Mgmt.(FRVM) eWIC card stock and inventory reporting Food cost containment Farmers Market Nutrition Program liaison SNAP Collaboration/FNS Field Office/STARS Peer Group Pricing Management The Integrity Profile Report Vendor Advisory Council lead Rebate contract lead Vendor Training
Idamaris Rodriguez Health Program Associate T: (860) 509-7197 F: (860) 509-8391 E-mail: Idamaris.rodriguez@ct.gov	Contract liaison for all WIC contracts; local agency and vendor management Contract lead for Infant Formula Rebate contract and Compliance Investigation contract Compliance purchase report reviews Vendor agreement/Sign off on warning letters Provides technical assistance to WIC vendors State Plan updates Prepares Annual training document for vendors Arranges and provides vendor trainings
Rafael Lima Health Program Assistant 2 T: (860) 509-7815 F: (860) 509-8391 E-mail: Rafael.lima@ct.gov	Online Monitoring for trafficking of WIC foods Participant and Retailer Fraud Investigations Vendor monitoring Vendor technical assistance
Marangelie Ortiz Health Program Assistant 2 T: (860) 509-7526	Compliance Investigations Compliance Buys Vendor complaints

F: (860) 509-8391 E-mail: Marangelie.ortiz@ct.gov	Vendor training High Risk Criteria Vendor monitoring as needed
Eric Marszalek Health Program Assistant 2 T: (860) 509-8072 F: (860) 509-8391 E-mail: Eric.marszalek@ct.gov	CT-WIC Vendor Portal Coordination Vendor Administrative Review Process A50 determinations WIC contact for EBT contractor Open enrollment and renewal process Technical assistance to vendors WIC webmaster
Beverley Daley Health Program Assistant 1 T: (860) 509-8076 F: (860) 509-8391 E-mail: Beverley.daley@ct.gov	Vendor monitoring Complaint follow up Vendor technical assistance
Barbara Quiros Health Program Assistant 1 T: (860) 509-7413 F: (860) 509-8391 E-mail: barbara.quiros@ct.gov	Competitive and Not to Exceed Pricing WIC Food Redemptions APL Maintenance Vendor training Technical assistance to Vendors and participants (transaction issues)
Maria Reyes Processing Technician T: (860) 509-7488 F: (860) 509-8391 E-mail: maria.reyes@ct.gov	Vendor application processing Vendor Authorizations Price Stock Survey updates Vendor correspondence/notification Tracks vendor penalties and prepares sanctions FNS/SNAP Collaboration/STARS
Susan Hewes Epidemiologist 3 T: (860) 509-7795 F: (860) 509-8391 E-mail: susan.hewes@ct.gov	Outcome objective analysis Program data analysis Produce results for quarterly objectives Internal/external data requests Adequate participant access determinations
Lauren Kramer Secretary 2 T: (860) 509-7462 F: (860) 509-8391 E-mail: lauren.conroy@ct.gov	State staff support Customer Service Order and maintain supplies Timekeeper Meeting minutes & training evaluations WIC Materials Management
Basil Obute Technical Analyst 2 T: (860) 509-7690 F: (860) 509-8391 E-mail: basil.obute@ct.gov	Systems development coordinator Local Agency technical support CT-WIC monitoring of automated processes CT-WIC data requests
Waqas Farooq Technical Analyst 2 T: (860) 509-7429 F: (860) 509-8391 E-mail: waqas.farooq@ct.gov	Development & maintenance Security Help Desk WICSmart WICShopper OCN

	Reporting
Deano Damico Technical Analyst 2 T: (860) 509-7688 F: (860) 509-8391 E-mail: deano.damico@ct.gov	Help Desk/Technical Support Equipment Prep/Deployment/Inventory Hardware Maintenance/CT-WIC update Hardware/Software Purchase
Michael Colello Technical Analyst 2 T: (860) 509-7210 F: (860) 509-8391 E-mail: michael.colello@ct.gov	Help Desk/Technical Support Equipment Prep/Deployment/Inventory Hardware Maintenance/CT-WIC update Hardware/Software Purchase
Colleen Grady Associate Accountant T: (860) 509-7709 F: (860) 509-7227 E-mail: Coleen.Grady@ct.gov	Financial Management of WIC grant Food Cost Estimation Review and monitor funding levels Organize and maintain budget. Project expenditures-budgets Work with auditors and program to ensure information reported is correct Monthly 798 reports for USDA Reconcile bank and treasurer accounts Monitor Local Agency cash flow, disbursements, and expenses
Yussuf Gulaid Fiscal Administrative Officer T: (860) 509-7713 F: (860) 509-8391 E-mail: Yussuf.Gulaid@ct.gov	Bank reconciliation & Treasury Report Abbott rebates Local agency budgeting and expenditures Breastfeeding Peer Counseling EBT report 425

C. LOCAL AGENCY ORGANIZATION

LOCAL AGENCY	COORDINATOR	PROGRAM NUTRITIONIST
Bristol Health 9 Prospect Street Bristol, CT 06010 (860) 585-3280 mdickau@bristolhospital.org	Melissa Dickau	Christine Marschall
Connecticut Institute for Families (CIFIC) 120 Main Street Danbury, CT 06810 (203) 797-4629 evansa@ct-institute.org	Amanda Grande	Molly Ruhan
Day Kimball Healthcare, Inc. 320 Pomfret Street Putnam, CT 06260 (860) 928-6541 KMorrisette@DayKimball.org	Kera Morrisette	Bianca Grover
East Hartford Health Department 754 Main Street East Hartford, CT 06108 (860) 291-7323 pmascoli@easthartfordct.gov	Patricia Mascoli	Carley Bedel
Hartford Health Department 131 Coventry Street Hartford, CT 06112 (860) 757-4780 shana.brierley@hartford.gov	Ponita Khouy	Ponita Khouy
Meriden Health Department 165 Miller Street Meriden, CT 06450 (203) 630-4245 scarpenter@meridenct.gov	Shelley Carpenter	Dana LaMontagne
Optimus Health Care 1450 Barnum Avenue Bridgeport, CT 06610 (203) 333-9200 vsantiago@opthc.org	Verletha Santiago	Lauren Keenan
Thames Valley Council for Community Action (TVCCA) 83 Huntington Street New London, CT 06320 (860) 425-6620 rbrady@tvcca.org	Regina Brady	Jamie Shannon
Yale New Haven Hospital Saint Raphael Campus	Deborah Diehl	Jennifer Gemmell

150 Sargent Drive, 2 nd Floor New Haven, CT 06511 (203) 688-7670 Deborah.Diehl@ynhh.org		
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PROGRESS ON

FFY 2024

GOALS AND OBJECTIVES

Program Functional Area 1: Management and Organization

Goal 1: Ensure program integrity, cost-effectiveness, quality of services and accountability in the delivery of nutrition services in the provision of food benefits.

By September 30, 2024

Objective 1.1: Convene monthly meetings (in-person or virtual) for local agency coordinators to ensure on-going communication and feedback loop.

Objective 1.2: Actively participate in MyCT project in 2024 to improve ease of access for applicants and ensure compliance with confidentiality regulations.

Objective 1.3: If funding is available, sponsor at least one leadership workshop for local agency management staff).

Objective 1.4: Update DPH and WIC Continuity of Operations Plan (COOP) to include recommended USDA components and ensure alignment.

Objective	Strategies/Activities	Baseline	Indicators	Progress
1.1 Convene monthly meetings (in-person or virtual) for local agency coordinators to ensure on-going communication and feedback loop.	Schedule and facilitate at least 10 meetings with Program Coordinators to: - Review policies and procedures. - Discuss funding and staffing issues. - Review CT-WIC data availability and reports. - Other topics as determined.	N/A	Summary of meetings. Improved compliance with policies and procedures. Increased sharing of strategies regarding resource utilization and staff retention. Improved use of data and discussions in LAP.	A total of 10 Program Coordinators' meetings were convened via MS Teams in FY2024 to keep local agencies updated on funding; policies and procedures; formula availability; data management; the transition to electronic Farmers Market Nutrition Program benefits and more. Meetings provided an opportunity for local agency staff to share information on resource utilization, program operations, and other key topics. State agency staff and partners presented at meetings on CT-WIC MIS system and other IT updates, partner referral opportunities such as CT Paid Leave, and on research partnerships such as Today's Baby.
1.2 Actively participate in MyCT project in 2024 to improve ease of access for applicants and ensure compliance with confidentiality regulations.	- Attend meetings. - Provide clarification and information regarding WIC Program Regulations and policies. - Develop MOU/MOA as needed	N/A	WIC Program successfully integrated into MyCT project and application.	The WIC Director and State Nutrition Coordinator, as well as other relevant state agency staff attended regular meetings in FY2024 with the MyCT project team to finalize development efforts and prepare for launching the first phase of the site, which

				occurred in October 2023. Connecticut WIC Program (CT WIC) staff continued to work with myCT developers to address issues, make updates, and ensure seamless running of the new site. The website is available here: https://health.ct.gov/ and the press release by Gov. Lamont is available here: https://portal.ct.gov/office-of-the-governor/news/press-releases/2023/10-2023/governor-lamont-announces-launch-of-health-ct-gov. CT WIC staff will continue to work with developers to maintain the site, address bugs, and ensure updates are made as needed. In addition, CT WIC staff worked with developers to create content for a chatbot that will accompany the Health.ct.gov website. The launch is planned by end of summer 2024.
1.3 If funding is available, sponsor at least one leadership workshop for local agency management staff.	• Determine if funding is available. • If yes, proceed with identification of topic for workshop. • Identify speaker and location. • Complete workshop and conduct evaluation for feedback.	N/A	Courses offered and successfully completed.	Because several leadership courses were offered in FFY2023, it was decided instead to focus on celebrating 50 years of WIC, including where we have been, and where we are going, at a local agency management meeting. A meeting was convened in person with all local agency management staff on March 27, 2024. Key topics included discussion of the state agency's vision for WIC services and a viewing of parts of the Minnesota WIC Press Conference featuring Secretary Vilsack; review of the multi-state participant survey results; identification of specific local agency focused strategies to address survey results; and a best practices

				presentation by one local agency coordinator. Local agency management were tasked with taking back their identified strategies for improvement to discuss with all staff and develop a plan for the Local Agency Showcase, which will occur at the statewide WIC 50th Anniversary Event in Sept 2024. The Local Agency Showcase will be an opportunity for local agencies to highlight something positive about their region and the work they do for WIC.
1.4 Update DPH and WIC COOP to include recommended USDA components and ensure alignment.	• Work with DPH's Public Health Preparedness and Local Health Section to review existing COOP and identify gaps. • Update as appropriate.	N/A		The State WIC Director, State Nutrition Coordinator, and FRVM Supervisor met with DPH Public Health Preparedness to review the plan for updating DPH's COOP and incorporating WIC as an essential function within the agency. A Business Process Analysis has been drafted, with a flow chart and submitted for review. This will continue into the next fiscal year, as the DPH COOP is still in process. A draft version is included in this submission. The State Agency Internal Operating Procedures Plan for State WIC staff has been drafted to include additional details and ensure compliance with new AFBA requirements. This document is also included with this submission.

Program Functional Area 2: Nutrition Services, Breastfeeding Promotion and Support*

Goal 2: Improve the nutritional and overall health of WIC families in Connecticut.

By September 30, 2024,

Objective 2.1: At least 35% of pregnant women participating in the WIC Program for a minimum of 6 months gain appropriate weight.

Objective 2.2: The incidence of low birth weight (LBW) among infants whose mothers were on the WIC Program for at least six months during pregnancy does not exceed 4%.

Objective 2.3: The prevalence of anemia among children enrolled in the WIC Program for at least one year does not exceed 7.5%.

Objective 2.4a: The prevalence rate of BMI $\geq$ 85th percentile to < 95th percentile for children 2-5 years does not exceed 15%.

2.4b: The prevalence rate of BMI $\geq$ 95th percentile for children 2-5 years of age does not exceed 10%.

Objective 2.5: At least 80% of infants enrolled in the WIC Program (have mothers who) initiate breastfeeding.

Objective 2.6: At least 60% of infants enrolled in the WIC Program are breastfed for 6 months or more.

Objective 2.7: Monitor the maintenance of CT-WIC via policy revisions, provision of State and local agency staff training and technical assistance, update of CT-WIC user manuals/helpfiles and participant training materials.

Objective 2.8: Local agency liaisons will make at least 1 in-person visit or phone call to each local agency to provide technical assistance in FY 2024.

Objective	Strategies/Activities	Baseline	Indicators	Progress
2.1 At least 35% of pregnant women participating in the WIC Program for a minimum of 6 months gain appropriate weight.	Through State MER chart audits and observations investigate baseline data for MWG indicators to determine areas for local agency improvement and recognize areas of local agency competence. Review of local agency LAPs and show that few local agencies have implemented the Tell Me More (TMM) About You tool in prenatal appointments in FY23. As a result, the State agency developed a required strategy for local agencies to use TMM About You in FY24. A baseline will be established using results reported in FY25 LAPs. The State agency will facilitate collaboration among Program Nutritionists in the last quarter of	2024 WIC Objective: $\geq$ 35% <u>FFY 2019 Target:</u> $\geq$ 35% Average: 27.7% Range: 13.5% - 46.3% <u>FFY 2020 Target:</u> $\geq$ 35% Average: 26.5% Range: 7.1%-40.2% <u>FFY 2021 Target:</u> $\geq$ 35% Average 27.0% Range: 6.9-74.3% <u>2022 Target:</u> $\geq$ 35%* Average 27.5% Range: 4.9-42.9% 2023 Target: $\geq$ 35%* Average 28.3% Range: 11.6% - 47.5% 2024 data pending. <u>Data Sources:</u> FFY2019-2021: CT-WIC MIS (Management Information System).	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports. MER chart reviews and Observations. Improvement in trend data over time for low performing agencies.	There has been some steady improvement in the statewide average (28.3%) and range for the MWG objective. Prenatal weight gain recommendations were documented 70% of both charts reviewed and observations. One of three agencies reviewed in FY24 struggled with adequately discussing weight gain recommendations during observations and failed to appropriately document. In one local agency while staff reviewed weight gain recommendations there is limited evidence in documentation that staff partnered with participants to establish goals to ensure

	FY23, and first quarter of FY24, to develop training, implementation, and evaluation process for the FY 2024 strategy. When indicated, local agency liaison, may provide local agency management with feedback on CAP response and/or implementation via technical assistance visits, quality assurance activities (chart audits or observations) and/or training related to deficiencies in MWG indicators.	*Beginning with 2022 State Plan submission, we will report available complete calendar year data.		appropriate maternal weight gain was achieved. One of the three agencies reviewed is utilizing the TMM prenatal tool as evidenced in both chart audit review and observations. Staff would benefit from continued training in the effective utilization of this tool to support healthy maternal weight gain. Overall, local agency staff would benefit from training on assisting participants in and documentation of a facilitated action plan for appropriate maternal weight gain.
2.2 The incidence of low birth weight (LBW) among infants whose mothers were on the WIC Program for at least six months during pregnancy does not exceed 4%.	Through State MER observations and review of documentation in chart audits, 80% of local agency staff incorporate prenatal weight gain guidance concepts in counseling, provide timely referrals and offer appropriate education materials that address applicable <i>modifiable</i> risk factors: - Smoking - substance use (including prescription drugs) - Prenatal weight gain - Mental health concerns (PMAD) Investigate Local Agency Plan (LAP) community needs assessments to determine baseline of LAP's that include data and/or add context to variables that impact LBW deliveries:	2024 WIC Objective: ≤ 4% FFY 2019 Target: ≤ 6.0% Average: 2.5% Range: 0.0% - 13.6% FFY 2020 Target: ≤ 6.0% Average: 2.7% Range: 0.0% - 13.8% FFY 2021 Target: ≤ 6.0% Average: 3.1% Range: 0.0% - 9.5% 2022 Target: ≤ 4.0%* Average: 3.1% Range: 0.0% - 12.5% 2023 Target: ≤ 4.0%* Average: 4.7% Range: 0% - 17.2% 2024 data pending.	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports. Improvement in trend data over time for low performing agencies.	This objective was not met this year, likely due to one quarter with unusually high result. Of the three agencies reviewed to date, 70% of records indicated some discussions on prenatal weight gain guidance. In 70% of observations/chart audits staff provided timely referrals for smoking cessation and other substance use programs. One local agency failed to document provision of referrals to participants who indicated substance use. 70% of records indicate the PMAD screen is

	• teenagers • substance use • income • ethnicity • geographic location • mental health Maintain relevant State agency collaborations that impact this outcome objective. (See Functional Area X)	FFY2019-2021: CT-WIC MIS (Management Information System). (*excludes pre-term & multiple births) *Beginning with 2022 State Plan submission, we will report available complete calendar year data.		conducted during the appropriate time frame and resources are provided appropriately. One of the three agencies reviewed had significant findings related to the administration of the PMAD screen. Findings included staff not asking staging questions to identify the appropriate screen (3 Question or 10 Question), staff failed to appropriately describe or introduce the screen to the participant, and PMAD screens were administered verbally over the phone.
2.3 The prevalence of anemia among children enrolled in the WIC Program for at least one year does not exceed 7.5%.	Through State MER observations and chart audits 90% of local agency staff effectively provide education to parents to reduce risk of development of anemia including: ▪ Importance of timely blood work, ▪ Risk of lead poisoning ▪ Making appropriate referrals and follow-up. Local agency liaisons will perform anemia QA chart audits in FY24. A total of 15 chart audits (sample to include 5 – 9-month infants; 5 – C1; and 5 – C2) to determine completeness of nutrition education documentation. Certain agencies will be selected based on outcome indicators. Indicators include anemia prevention, nutrition assessment, lab	2024 WIC Objective: ≤7.5% FFY 2019 Target: ≤ 7.5% Average: 8.5% Range: 3.9% - 19.0% FFY 2020 Target: ≤ 7.5% Average: 8.5% Range: 3.5% - 18.2% FFY 2021 Target: ≤ 7.5% Average: 8.8% Range: 2.4% - 17.5% 2022 Target: ≤ 7.5%* Average: 8.2% Range: 2.9% - 12.7% 2023 Target: ≤ 7.5%* Average: 8.6% Range: 2.9% - 12.7% 2024 data pending.	CT-WIC quarterly and annual reports. Chart audits and observations. Change in trend data over time for low performing agencies.	The State agency did not meet the target for the anemia objective. The statewide average and range are stable. 90% of local agencies monitored are using effective educational methods and appropriate education materials to assist parents in prevention of iron deficiency anemia for their children. Of the two local agencies reviewed to date, one agency had a significant number of findings related to missing bloodwork. In the one agency reviewed, in 25% of charts audited there is missing bloodwork.

	results, iron supplementation and type of anemia (if indicated).	FFY2019-2021: CT-WIC MIS (Management Information System). *Beginning with 2022 State Plan submission, we will report available complete calendar year data.		In addition, in 10% of charts despite bloodwork on file staff failed to enter results in CT-WIC. The State agency performed quality assurance (QA) using child charts from four local agencies and with a range of ages. Local agencies achieved the best results on the following indicators: accurately entering H/H into CT-WIC (53%) and scheduling and completing follow-up visits (66.7%). Local agencies partially met or completely met the standards for having a complete assessment note (78.6%). Local agencies did not meet the standards for setting an action plan/goal (60%) and identifying factors for anemia (80%).
2.4 a. The prevalence of BMI $\geq$ 85thile to < 95thile for children 2-5 years of age does not exceed 15%. b. The prevalence of BMI $\geq$ 95thile for children 2-5 years of age does not exceed 10%.	Review of local agency LAPs and show that few local agencies have implemented the TMM About Your Child tool. As a result, the State agency developed a required strategy for local agencies to use TMM About Your Child in FY24. All child participants at risk of overweight or obesity will have the TMM questionnaire completed at their follow-up visits to assess behaviors and monitor progress.	<u>2024 WIC Objectives:</u> Overweight: $\leq$ 15% Obesity: $\leq$ 10% a. OVERWEIGHT: BMI $\geq$ 85thile to <95thile <u>FFY 2019 Target:</u> $\leq$ 15.0% Average: 15.1% Range: 10.1% - 20.1% <u>FFY 2020 Target:</u> $\leq$ 15.0% Average: 15.1% Range: 9.1% - 20.8% <u>FFY 2021 Target:</u> $\leq$ 15.0% Average: 14.7%	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports. Indicators of effective use include documentation of TMM, MI, Stages of Change and/or development of a plan.	The statewide target for the overweight objective was met, however, the obesity target was not met. Both overweight and obese targets show progress in the average and range. The State agency performed quality assurance on a total of 18 charts (2) for each local agency. Local agencies achieved the best results on the following indicators:

	A baseline will be established using results reported in FY25 LAPs. The State agency will facilitate collaboration among Program Nutritionists in the last quarter of FY23, and first quarter of FY24, to develop training, implementation, and evaluation process for the FY24 strategy. The State agency will provide a presentation on the results of the FY23 TMM chart audits by December 2023 and identify areas for improvement in counseling and documentation for children at risk of overweight/obesity. By the third quarter of FY24, the State agency will determine the need to update and convert the Child TMM tool to a fillable form. Local agency liaisons will perform TMM QA chart audits in FY 2024. A total of 15 chart audits (sample to include child participants with at risk for or with overweight or obesity risk) to determine completeness of nutrition education documentation. Certain agencies will be selected based on outcome indicators. 50% of local agencies continue to use WICSmart modules for obesity/overweight prevention. This will be assessed in routine monitoring.	Range: 8.6% - 22.9% <u>2022 Target:</u> ≤ 15.0%* Average: 14.4% Range: 10.0% - 18.6% <u>2023 Target:</u> ≤ 15.0%* Average: 14.7% Range: 8.6% - 20.2% b. <u>OBESITY:</u> BMI ≥ 95thtile <u>FFY 2019 Target:</u> ≤ 10.0% Average: 15.3% Range: 9.3% - 22.8% <u>FFY 2020 Target:</u> ≤ 10.0% Average: 15.5% Range: 7.7% - 28.3% <u>FFY 2021 Target:</u> ≤ 10.0% Average: 19.1% Range: 12.5% - 28.1% <u>2022 Target:</u> ≤ 10.0%* Average: 16.7% Range: 8.8% - 23.1% <u>2023 Target:</u> ≤ 10.0%* Average: 15.8% Range: 10.5% - 21.2% 2024 data pending. FFY2019-2021: CT-WIC MIS (Management Information System). *Beginning with 2022 State Plan submission, we will report available complete calendar year data.		28% of charts reviewed had documentation of TMM to start a conversation. 67% and 69% of charts audited assessed for snacks and sweetened beverages respectively. 70% of charts assessed for physical activity. 40% of charts assessed showed evidence of validating anthropometrics. 75% of charts audited had Care Plans that documented nutrition counseling. 36% and 16% of charts audited had specific nutrition or physical activity plans documented, respectively. The State agency provided a presentation on the FY 2023 chart audits for the Program Nutritionists. Another presentation will be provided in December 2024 with the results of current overweight/obesity chart audits. To date in FY24 there was a 60% increase in WICSmart module completion over 2023 levels. Six of the nine local agencies have notable improvement in WICSmart
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				completion from FY23 to FY24. Of the three agencies reviewed to date, only one agency is utilizing WICSmart as an appointment option. However, there is little indication that WICSmart is provided as a strategy for the childhood overweight/obesity objective. Conversations continue with management to support assignment of WICSmart as 1.) an option for children with overweight/obesity and 2.) as a caseload management tool.
2.5 At least 80% of infants enrolled in the WIC Program (have mothers who) initiate breastfeeding.	Through State MER observations and chart audits 50% of local agency staff will incorporate and document concepts from breastfeeding Content Sheets into prenatal education and counseling. Breastfeeding Unit to conduct 1-2 enhanced breastfeeding reviews at local agencies. (One review will include agency with peer program.) See 2.6.	2024 WIC Objective: ≥80% <u>FFY 2019 Target:</u> ≥ 70.0% Average: 81.6% Range: 59.9% - 94.4% <u>FFY 2020 Target:</u> ≥ 70.0% Average: 81.3% Range: 63.9% - 93.6% <u>FFY 2021 Target:</u> ≥ 80.0% Average 79.8% Range: 61.1% - 94.3 % <u>2022 Target:</u> ≥ 80.0%* Average: 79.6% Range: 62.8% - 94.9% 2023 Target: ≥ 80.0%* Average: 79% Range: 58.1% - 94.6% 2024 data pending.	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports, Chart Audits and Observations conducted during Enhanced Breastfeeding Reviews and Peer Counseling Monitoring. Results of enhanced Breastfeeding MERs, show documentation of 80% compliance with CT Guidelines for Breastfeeding Promotion and Support.	The breastfeeding initiation objective was not met, with a statewide average of 79%. Six of the nine local agencies had breastfeeding initiation rates of over 80%. The remaining had initiation rates ranging from 68%-79.4%. The State agency Breastfeeding Unit is hoping to see an increase in these rates based on implementation of Make a Plan strategies in FY 2023. Two of three local agencies reviewed to date in FY23 were observed utilizing effective and appropriate education materials to assist mothers in successful initiation of

	Review of local agency LAPs and nutrition services monitoring results show few local agencies have implemented the Make a Plan (MAP) checklist and documentation of specific actions to assist in breastfeeding success. As a result, the State agency developed a required strategy for local agencies to implement the MAP checklist during prenatal second contacts. Indicators include	FFY2019-2021: CT-WIC MIS (Management Information System). *Beginning with 2022 State Plan submission, we will report available complete calendar year data based on previous calendar year.	breastfeeding. The Nutrition Services monitor was able to validate that two of the three agencies reviewed utilize the Make a Plan (MAP) checklist during counseling. In one agency reviewed there were several findings that indicated staff are not providing early and frequent breastfeeding education and support to women during the prenatal period. Additionally, in FY 2024 the one enhanced breastfeeding review conducted showed 70% compliance with CT Guidelines for Breastfeeding Promotion and Support. (Short of the 80% benchmark) During the monthly Breastfeeding Coordinator calls, the State agency Breastfeeding Unit focused on implementing targeted prenatal breastfeeding counseling at second contacts to ensure participants meet their established feeding goals. Local agency breastfeeding coordinators worked together to develop guidance for training, implementation and quality assurance for the LAP breastfeeding initiation strategy.
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	documentation of preparing for hospital, family/friend support, skin-to-skin contact, rooming-in and community support. A baseline will be established using results reported in FY25 LAPs. The State agency will facilitate collaboration among Breastfeeding Coordinators in the last quarter of FY23, and first quarter of FY24, to develop training, implementation, and evaluation process for the FY24 strategy. Breastfeeding Unit (State Breastfeeding Coordinator and State agency WIC Peer Counselor Coordinator (IBCLC)) continue to participate in Regional WIC Breastfeeding Curriculum training (CT, NH, ME, VI and VT) to train local agency staff and select community partners using the USDA/FNS DBE Curriculum. If awarded, contribute to SPAN 0012 breastfeeding activities related to initiation: Ten Step Collaborative, Ready, Set Baby webpage. Diversity in Lactation, and update of It's Worth it Campaign.			The Regional WIC Breastfeeding Training Curriculum continued for FY 2024. Most of the staff trained were from Connecticut WIC and staff from several community partners such the Community Health Network of CT (CHN-CT) and home visiting programs. Unfortunately, while CT-DPH's SPAN 0012 application was approved, funding wasn't available. Where possible, these breastfeeding strategies will continue with other identified funding sources.
2.6 At least 60% of infants enrolled in the WIC Program are breastfed for 6 months or more.	Through State MER observations and chart audits 50% of local agency staff will incorporate and document concepts from 7 duration focused breastfeeding content sheets, HUSKY breast pump access, and overview of CT breastfeeding	2024 WIC Objective: $\geq 60\%$ <u>FFY 2019 Target:</u> $\geq 50\%$ Average: 65.2% Range: 37.5% - 90.7% <u>FFY 2020 Target:</u> $\geq 50\%$ Average: 64.4% Range: 42.6% - 88.9%	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports. Chart audits and Observations.	During FY 2023-2024 the State agency has been conducting quality assurance (QA) on all data reports. The breastfeeding duration reports, and target will need significant changes based on the QA.

	laws, into individual or group prenatal counseling/education.	<u>FFY 2021 Target:</u> ≥ 60% Average: 61.8% Range: 38.6% - 89.3% <u>2022 Target:</u> ≥ 60%* Average: 62.7% Range: 37.7% - 90.4% 2023 data pending. FFY2019-2021: CT-WIC MIS (Management Information System). *Beginning with 2022 State Plan submission, we will report available complete calendar year data.		We hope to have a revised breastfeeding duration target for FY 2026. In two of the three agencies reviewed there is evidence that local agency staff are utilizing the concepts from the breastfeeding content sheets, HUSKY breast pump access, and overview of CT breastfeeding laws, into individual education sessions. There is evidence that staff are utilizing the Make it Work checklist during counseling in 10% of charts audited. During the monthly Breastfeeding Coordinator calls, the State Breastfeeding Unit focused prioritizing breastfeeding follow-up appointments (BF-F/U) to manage immediate feeding challenges. Local agency breastfeeding coordinators worked together to develop guidance for training, implementation and quality assurance for the LAP breastfeeding duration strategy. Local agencies submitted their BF-F/U, referral and triage plans to the State Breastfeeding Unit for review and feedback. This was completed by November 2023.
	Local agencies do not have daily CT-WIC schedules that include access to timely breastfeeding follow-up appointments. As a result, the State agency developed a required strategy for local agencies implement in FY24, to include the addition of daily, non-benefit issuance, breastfeeding follow-up appointments and to develop and train staff on a local agency referral and triage protocol/process. Based on results reported in FY25 LAP, determine training needs or adjustments to polices.			

	The State agency will facilitate collaboration among Breastfeeding Coordinators in the last quarter of FY23, and first quarter of FY24, to develop training, implementation, and evaluation process for the FY24 strategy. Provide oversight and technical assistance to 7 WIC clinic based Breastfeeding Peer Counseling Programs (BFPC) and one hybrid program through on-site visits, conference calls and review of quarterly program and financial reports.		Quarterly activity and expenditure reports from peer counseling contractors, including # of women enrolled, and duration rates are reviewed and approved. CT-WIC Peer counseling modules are being used	CT presented on the implementation of the BF-F/U appointments during a panel discussion at the May 29th NERO Nutrition and Breastfeeding Coordinators Meeting. Preliminary results from local agency QA show between 90-100% show rate for BF-F/U appointments and reported that staff are feeling more confident in their ability to assist families and more job satisfaction. Additionally, while not all participants that complete a BF-F/U appointment continue breastfeeding, local agencies report participants are likely breastfeeding longer than they otherwise would, and some families have maintained or increased intensity of breastfeeding. See the FY 2024 Breastfeeding Peer Counseling Implementation Plan for details related to technical assistance and oversight for the WIC BFPC Program. See Breastfeeding Initiation strategy for details about SPAN cooperative agreement.
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	Continue to monitor implementation of peer-counseling module in CT-WIC development of consistent, automated data reports for peer counseling programs. If awarded, contribute to SPAN 0012 breastfeeding activities related to duration: increasing access and equity in lactation, Collaboration with PRAMS Epidemiologists on Data to Action documents, Workplace Lactation Accommodations, and Breastfeeding Friendly Medical Offices Recognition.		appropriately in agencies with peer counseling programs. SPAN 0012 reporting.	
2.7 Monitor the maintenance of CT-WIC via policy revisions, provision of State and local agency staff training and technical assistance, update of CT-WIC user manuals/helpfiles and participant training materials.	Facilitate weekly internal (DPH) CT-WIC support meetings for FY24 to improve CT-WIC performance and reduce CT-WIC problems. Develop Release Notes for scheduled CT-WIC releases. Determine new process for monitoring nature of Help Desk calls (immediately following a Release) to provide targeted training or QA needs.	Help Desk Calls.	Reduction of Helpdesk calls from local agencies related to CT-WIC policies and transaction information requests.	Weekly internal DPH CT-WIC support meetings were held in FY24. WIC IT, Nutrition, and FRVM staff were in attendance. Meetings focused on troubleshooting bugs and the maintenance of CT-WIC. The majority of FY24 MIS activities were focused on development changes to the CT-WIC Finance module. These changes will allow local agency management and financial administrators to enter budgetary information into the MIS directly rather than using spreadsheets. The updates to the Finance module and other bug fixes will be included in CT-WIC Release 4.2 scheduled in August 2024. Release notes and Memorandums

				will be developed for state and local agency staff. The DPH Helpdesk did not implement new software for reporting issues. Local agency staff continue to call or email the DPH IT Helpdesk to report issues. The numbers below represent DPH IT Helpdesk tickets opened, resolved, and closed. FY23 (10/1/22-6/20/23) – 1770 Helpdesk tickets FY24 (6/21/23-6/20/24) – 1782 Helpdesk tickets
2.8 Local agency liaisons will make at least 1 in-person visit or phone call to each local agency to provide technical assistance in FY 2024.	Liaisons will meet at least 2 times in FY24 to discuss tracking spreadsheet and local agency trends observed.		Updated LAP outcomes summary tool During technical assistance visits, local agency liaisons will discuss the local agency's progress at achieving its measurable strategies for all nutrition outcome objectives. Trends or issues identified at liaison meeting/discussion 2x/year.	Liaisons met internally three times in FY 2023 to discuss LAP checklist and overall trends from liaison visits. Significant technical assistance was provided due to the change in host agency contractors for several local agencies in FY 2024 and preparation for another host agency change in FY 2025. In addition, one local agency had Program Coordinator turnover which required additional liaison focus.

±See Functional Area 12, Data Quality, Analysis and Reporting for information Epidemiology and IT specific activities related to these strategies.

Program Functional Area 3: FOOD DELIVERY AND FOOD INSTRUMENT ACCOUNTABILITY

Goal 3: To improve food delivery operations at the state and local agency level.

Objective: 3.1 Improve functionality of the Nutrition Module in CT-WIC.

Objective: 3.2 Follow up on participant shopping experience project.

Objective: 3.3 Cross training of staff on vendor and nutrition modules.

Objective: 3.4 Support of Department of Agriculture in electronic Farmers Market Nutrition Program (FMNP).

Objective: 3.5 Make changes to food packages and eligible foods in response to Final Rule for Revisions in the WIC Food Packages.

Objective	Strategies/Activities	Baseline	Indicators	Progress
3.1 Improve functionality of the Nutrition Module in CT-WIC.	Utilize redemption data to determine the status of authorized vendors in meeting the competitive pricing criteria.	October 2022	Manual exports are needed to obtain the data. Prices collected during monitoring visits do not reflect items that participants are purchasing.	Competitive pricing report is now being tested in the SSRS reporting portal and will be used monthly to determine if vendors are competitively priced.
	Correct the functionality of UPC Upload for batch uploading into the APL.	October 2018	CT-WIC errors occurred when uploading text file.	WIC IT staff make batch changes in the database. Will explore in Q1 of FY25 how to populate the database using a spreadsheet with new APL items.
	Assess NTE Modeling enhancement and develop cat/subcat max price report in reporting portal.	August 2021	Reports in SSRS reporting portal utilize data from CTWIC.	Updates were made to the cat/subcat report in February 2024 and is available in the SSRS reporting portal. CT WIC is using the 97% of Max Price report to assess high

				prices that are close to the NTE.
	Implement the high price indicator functionality in CT-WIC to accommodate maximum prices for items higher in price due to alternate packaging (i.e. multi-packs of yogurt, half gallons and quarts of milk, etc.)	August 2021	Reports in SSRS reporting portal utilize data from CTWIC.	High Price indicator was implemented on quarts and half gallons of milk in January 2024, and on eggs in March 2024 due to alternate types of eggs now available.

3.2. Follow up on participant shopping experience project.	New local agency staff will conduct WIC purchases with test cards to gain a perspective of the participant's experience in shopping.	October 2019	New employee orientation and onboarding for new staff. Improve understanding of WIC shopping challenges.	New cards were issued for Hartford and the new Waterbury service area, and benefits were updated for the Optimus sites.
	Standardized training program for local agencies to train participants on obtaining balance inquiries, looking for shelf labels, using a mobile app for WIC approved products.	October 2019	The creation of a standardized training program and consistent use of it, leading to fewer complaints and phone calls about participants not knowing how to use card.	During the shopping experience section of the Local Agency Orientations on 12/8/23 and 6/21/24, state staff presented on card activation, shopping with WIC, the Approved Food Guide, WICShopper app, and troubleshooting shopping issues.
	Utilize results of Participant Satisfaction survey to assess improvements in participants' shopping experiences.	FY21 data Results of NWA Participant Satisfaction Survey	Did not purchase options because they did not know how: 45.4%; Could not find WIC foods: 48.8%; Difficulty finding transportation: 20.8%; Difficulty finding childcare: 40.1%; Did not buy all their WIC foods because: online shopping was not available: 41.9% and curbside pick not available 40.7%; Ordering WIC foods for pick up: 59.7%; Ordered WIC foods for home delivery for additional out of pocket fee: 34%; Used self-checkout: 65.3%.	Comparable results of the NWA Multi-State Participant Satisfaction Survey 2023 were: Did not purchase options because they did not know how: 38.2%; Could not find WIC foods: 41.7%. Difficulty finding transportation: 13.8%; Ordered WIC foods for home delivery for additional out of pocket fee: 35.6%;

				Used self-checkout: 33.3%.
	Collaborate with Rhode Island and Vermont WIC Programs to explore the feasibility of online shopping for the Northeast States.	August 2022	State Directors discussion on assessing interest within our region and the common vendors we share. FY21 data regarding shopping from the NWA Multi State Participant Satisfaction Survey.	Monthly and adhoc meetings began on October 20, 2023, and continued discussions are exploring online shopping options for our states. VT has withdrawn from this collaboration project. West Virginia has been added to a similar project.
	Revise the Connecticut WIC Approved Food Guide with updated foods that are available in the APL for purchase			Revised WIC Approved Food Guide and Vendor cards were printed and distributed to all authorized WIC vendors in January 2024.
3.3 Cross training of staff on vendor and nutrition modules	Training of food resource and vendor management staff on connections between authorization, food items, pricing, monitoring, sanctions, reports, CT-WIC and EPPIC	July 2021	Improved knowledge of connections between CT-WIC modules and EBT provider portal	Category/Sub-category redemption report was presented to all FRVM staff on 10/4/23 to look at multiple areas of redemption, dates, families, vendors,

				products, and prices. Percentages of redemptions for the most frequently redeemed items were presented at the FRVM meeting on 4/15.
	Staff to provide walk throughs of CT-WIC screens to identify improvements and connections to enhance CT-WIC.	October 2021	Knowledge of WIC approved categories and subcategories, maximum prices, food issuances, food packages, peer group averages and assignments	Reviewing reports proved to be more helpful than the CT-WIC screens, and permissions to certain areas are limited because they are role based. Preliminary discussions are beginning for the changes to the CT-WIC screens for the new food package changes.
3.4 Support of Department of Agriculture (DoAg) in electronic Farmers Market Nutrition Program (FMNP).	Using the SoliPortal, provide assistance to families who contact the state or local offices with questions about receiving FMNP benefits.	July 2023	DoAg implemented eFMNP using SoliSystems Corporation in July 2023	DPH and DoAg have been meeting frequently since 1/24/24 to discuss currently active families in the contractor's system. Nations Benefits acquired Soli-Systems, and a revised data sharing MOU was executed on 7/2/24. DPH is working with DoAg and Nations Benefits to reconcile participant information and

				ensure all active, eligible participants receive their benefits.
	Collaborate with DoAg on messaging to participants about spending FMNP benefits through Q1 of FY24.	June 2023	Messaging began in June on the change to eFMNP vs. the use of paper vouchers	As of July 2024, a portion of FMNP benefits have been issued, and DPH is working with the local agencies to message to WIC participants that FMNP benefits are forthcoming.
3.5 Make changes to food packages and eligible foods in response to Final Rule for Revisions in the WIC Food Packages.	Changes to minimum inventory, pricing and monitoring reports will be needed. Review submissions from manufacturers and distributors on product nutrition labels, statewide availability, and retail costs	Announcement of the final rule.	Emails and information submitted to the State WIC Office for approval in Connecticut.	New food package (NFP) rule was made final on 4/18/24. A tracking spreadsheet is being utilized in Q2 to assess all the changes needed to all areas. An Infrastructure Grant to plan for and implement the NFP changes was applied for on 7/5/24.
	If needed, amendments to the vendor agreements to update the minimum inventory requirements.	Announcement of the final rule.	Agreements are revised and emailed to all authorized vendors.	See above.
	If needed, provide bulletin on new regulations or procedures.	Announcement of the final rule.	Bulletins sent to all WIC authorized vendors.	See above.
	If needed, communicate necessary changes in food packages to IT.	Announcement of the final rule.	Changes tested.	See above.
	If needed, provide updates to local agencies regarding any new policies, educational materials, etc.	Announcement of the final rule.	Numbered memo sent.	See above.

	If needed, notify wholesale distributors of updates to food packages, new food items, and minimum inventory requirements.	Announcement of the final rule.	Mailing list.	See above.
	Assess percentage of category redemptions to determine any increases in redemptions as a result of the new food package revisions.	Announcement of the final rule.	Report-Redemptions by Site and Food Category	See above.

Program Functional Area 4: Vendor Management

Goal 4: To improve communication and effectiveness in Vendor Management.

Objective: 4.1 Improvements in reporting vendor data and producing the extracts for FDP.

Objective: 4.2 Potential transition of EBT Processors

Objective: 4.3 Develop policies and establish business processes for a Policy and Procedure Manual.

Objective: 4.4 CTWIC Enhancements to streamline processes

Objective: 4.5 Training videos to supplement interactive vendor training

Objective: 4.6 Improve investigation services through RFP process and inventory audits.

Objective	Strategies/Activities	Baseline	Indicators	Progress
4.1 Improvements in reporting vendor data and producing the extracts for FDP.	Utilize CTWIC data to improve reporting of high risk vendor criteria, transaction level redemption data, redemption patterns, and produce the redemptions template for FDP.	June 2022	Refining reports in the SSRS reporting portal using the data from CTWIC to provide more comprehensive information.	CT WIC staff worked with a consultant from 1/17/24-6/30/24 to complete several of these reports, including the high risk vendor report that is being used in Q4. CT WIC staff worked with IT staff and completed this FDP report.
	Utilize CTWIC data to improve tracking of compliance investigations and produce the investigations template for FDP.	June 2022	Tracking the status of individual investigations and Key Performance Indicators. Revisions to high risk vendor report	CT WIC staff worked with a consultant until 6/30/24 to build the parameters of this report. This report will be used for FY24 FDP reporting.
	Utilize CTWIC data to improve tracking of violations and produce the violations template for FDP.	June 2022	Information is logged in CTWIC on the violations screen, and separate Excel spreadsheet is used for tracking purposes.	CT WIC staff worked with a consultant until 6/30/24 to build the parameters of this report. This report will be used for FY24 FDP reporting.

	Utilize CTWIC data to improve tracking for sanctions and produce the sanctions and claims collections template for FDP, in addition to referrals to SNAP on WIC DQs.	June 2022	Information is logged in CTWIC on the sanction screen, and separate Excel spreadsheet is used for tracking purposes.	CT WIC staff worked with a consultant until 6/30/24 to build the parameters of this report. This report will be used for FY24 FDP reporting.
	Utilize CT-WIC data to improve tracking for training and produce the annual training and data template for FDP.	June 2022	Anticipated attendance for training; sessions, vendors assigned, and attendance. Interactive, High Risk, Annual training.	CT WIC staff worked with a consultant until 6/30/24 to build the parameters of this report. CT WIC staff worked with IT staff and completed this FDP report.
	Utilize CT-WIC data to fulfill requirements for FDP spreadsheets, including populating the FDE template.	June 2022	WIC-44 report from TIP website is used as the basis for the TIP report.	IT staff completed the FDP reports and continue to make formatting adjustments.

4.2 Potential transition of EBT Processors	Participate in joint SNAP/WIC conference calls to finalize Northeast Coalition of States (NCS) EBT Services.	Calls began January 7, 2021.	Discussion of any common areas between programs. publishing dates per WIC/SNAP timelines; October 2020.	CT attended NCS Joint WIC SNAP EBT monthly calls until 3/21/24. Beginning in April 2024, the NCS calls were separated by WIC and SNAP, but a CT staff person attends both to stay updated on both programs' activities in contracting and transitioning to FIS.
	CT staff on selection committee to participate on approval of contractor.	October 2021	Estimated dates per OTDA timelines: Bid submission May 13, 2021 ; Award made December 2021. Finalization of New York's contract.	On 9/13/23, CT WIC was notified that New York State Office of the State Comptroller OSC ruled in favor of the Office of Temporary and Disability Assistance (OTDA) upholding the results of the NCS RFP, and NY SNAP's contract with FIS was executed. After that time, CT WIC could begin discussions with FIS, which have been ongoing.
	Draft WIC Contract to Legal for review.	May 2021	DAS approval to join NY State Contract provided on June 21, 2019.	CT WIC staff and an IT Consultant as the Project Manager have met with a contracts specialist and an attorney at the Department of Administrative Services (DAS) throughout FY24 to discuss the contract and all exhibits for FIS.
	Notify WIC Vendor Advisory Council and authorized vendors of contractor selection.	December 2021	Vendor community informed on potential changes.	Expected to occur in FY25.
	Finalize and execute DPH WIC contract to partake in NCS contract.	February 2022	DAS approval to join NY State NCS RFP provided on June 21, 2019.	The contract is being handled through CT DAS, and not through DPH. As of July 2024, CT WIC is reviewing the contract, the

				Statement of Work, and updating NERO and SSO on the progress.
	Transition and Conversion plan for data, stand beside devices, retailer portal.	July 2022	Transition to new provider or platform	Expected to occur in FY25.
4.3 Develop policies and establish business processes for a Policy and Procedure Manual.	Document new processes in response to changes to current business practices.	March 2020	Changes to the WIC Vendor Agreement, access	Documents are being updated.
	Revise policies and establish business processes for a Policy and Procedure Manual.	Policies and procedures in writing.	Implementation of eWIC, utilization of CT-WIC, and the COVID-19 pandemic changed processes. Shared knowledge of processes.	Documents are being updated.
	Modify the CT WIC Manual for the Vendor Module based on changes and enhancements made.	April 2018	Vendor Module Manual updated last for version 1.9.	Vendor Portal User Guide was updated in May 2024 and posted on website.
	Open the application portal multiple times per year for new vendors to apply for WIC authorization.	October 2023	Open enrollment now offered for new stores only in January each year.	Vendor portal was opened in January 2024 for open enrollment and in February, May and June 2024 for multiple stores under the same ownership to apply for CT WIC authorization. Website language was updated on 2/14/24 to accept online applications outside of the January open enrollment period.
4.4 CTWIC Enhancements to streamline processes.	Enhancements to Vendor Module training screens to increase the functionality and connection between training, monitoring and competitive pricing phases with selection criteria verification.	August 2020	Enhancements were delayed until the revised 3Sigma contract was in place. Currently the relationship between enrollment and routine operations are separate.	The 3Sigma (now Voyatek) contract has not yet been approved but has been sent to USDA on 7/5/24 for review and meets determination.

	Refinement of the full monthly redemption report with EBT data through CTWIC. Report captures data from multiple modules and redemption tables.	Database only accessible through EPPIC and/or IT report requests.	EPPIC access is limited to the Coordinators, and not available on an as needed basis.	The category/subcategory redemption report was made available in February 2024 in the SSRS reporting portal. Parameters for dates, cats/subcats and products can be set and provides multiple areas of redemption such as transaction and settled dates, families, vendors, products, and prices paid.
	Utilize CTWIC via tablet to allow real-time access to vendor information and provide electronic record of monitoring visits.	Monitoring visits are currently done on paper.	Number of electronic documents available through the vendor portal to the MIS. Apple iPads can be used to access CTWIC for in-store use.	Currently, a MiFi device is being utilized with the Apple iPad to conduct monitoring visits but is not a long-term solution. The state is having a delay in obtaining SIM cards for the devices purchased and will continue to pursue their use.
4.5 Training videos to supplement interactive vendor training	Recordings through Microsoft Teams to develop training videos to be used for open enrollment and renewal trainings.	October 2019	Consistent messaging for all training sessions.	During an interactive training on Teams, staff used the "Present in Teams" option while showing a PowerPoint presentation. Teams will record just the voice and the slides, so CT WIC will continue to explore this option and integrate this into interactive training.

	Post on website for vendor use in training additional staff.	October 2019	Assistance to vendors to train more employees on WIC policies and procedures.	Video posting is postponed until attendance tracking can be achieved. In May 2024, CT WIC created a Training Documents section on the Retailers page of the WIC website and posted the current versions of the New and Renewal Vendor training presentations.
	Request from other states who utilize post-training tests, online assessments to test knowledge of training.	October 2019	Training chapters will apply to different areas regarding minimum stocking requirements, transaction processing, violations, and sanctions.	Information was obtained from other states in May 2024 on recorded trainings that meet the training requirements and vendor attendance surveys.
4.6 Improve investigation services through RFP process and inventory audits.	Re-Initiate an RFP for Compliance Investigation contract.	WIC timeline: RFP release date: 10/1/23.	Developed and issued an RFP for compliance buys investigation. No responses received in FY21; will revise and repost.	RFP was not initiated. CT DPH has experienced staffing shortages and significant backlogs in contracts management, fiscal and legal, which are areas that would be involved in the RFP and contracting process. Undercover investigations and inventory audits have continued to be conducted by State WIC Office staff.
	Screen and evaluate all proposals to determine if bidders meet the requirements of the RFP.	WIC Timeline: Tentative 12/1/23.	Reviewed RFP proposals.	RFP was not initiated, so activity was not conducted.
	Select a vendor to conduct compliance buys investigation services for the WIC program.	WIC Timeline: Tentative contract award notification: 12/8/23.	Screening team selected winner for the compliance buys investigation services.	RFP was not initiated, so activity was not conducted.
	Develop a contract language for compliance investigation services.	WIC Timeline: Tentative 1/1/24.	Developed new contract language-	RFP was not initiated, so activity was not conducted.
	Submit paperwork to initiate contract process.	WIC Timeline: Tentative 1/1/24.	Final contract paperwork submitted for review and approval.	RFP was not initiated, so activity was not conducted.

	Follow up contract process to make sure contract is executed.	WIC Timeline: contract Agreement start date: 10/1/24.	Executed contract with a selected vendor to provide compliance buys investigation services.	RFP was not initiated, so activity was not conducted.
	Explore the use of summer interns and those seeking non-DPH internships for students to conduct compliance investigations.	January 2024	Collaboration with criminal justice students or Connecticut Department of Emergency Services and Public Protection	FRVM staff attended two CT DPH Summer Internship Preceptor Q&A sessions in April 2024. It was determined that this work would not provide the public health practice experience that a DPH intern would require.

Program Functional Area 5: Management Information Systems

Goal 5: To maintain and enhance the WIC IT infrastructure through 9/30/2024.

Objective: 5.1 Implement solutions or new technologies to address changes in USDA regulations and/or state policy.

Objective: 5.2 Move towards a self-service reporting environment for regular WIC information needs.

Objective: 5.3 Develop a new MIS equipment obsolescence plan.

Objective: 5.4 Increase staff knowledge and utilization of current IT languages, tools and techniques.

Objective: 5.5 Implement new technologies to enhance productivity or system security.

Objective: 5.6 Implementation planning for next EBT contract

Objective: 5.7 Implement next MIS support contract

Objective: 5.8 Implement Participant Portal

Objective	Strategies/Activities	Baseline	Indicators	Staff Assigned	Progress
5.1 Implement solutions or new technologies to address changes in USDA regulations and/or state policy.	Add additional functionality to CT-WIC.	MIS-WIC implementation project complete.	Timely update of changes to reflect USDA policies.	WIC IT Staff	CT WIC is part of the state's myCT health and human service (HHS) portal, a one-stop online location to help residents determine benefit eligibility through a prescreener, and link existing HHS benefits between programs. The myCT feed for visibility of WIC was added as part of this effort to allow WIC participants to link their benefits in the portal. CT WIC, in partnership with DoAg, began e-FMNP in 2023, and continued to send participant data feeds to DoAg for the 2024 season. The State agency is working to implement a new finance module in

					the CT-WIC system which will allow for better local agency budgeting and expenditure reporting. Significant planning and development has occurred in the current fiscal year, with the plan for release/full implementation by 9/30/24.
5.2 Move towards a self-service reporting environment for regular WIC information needs.	Implementing data dashboard to replace file and paper distribution.	Webfocus reports have been converted to SQL Server Reporting Services (SSRS)	Webfocus reports sunset, begin conversion to PowerBI	WIC IT Staff WIC Epidemiologist Nutrition Consultant	Webfocus reports are converted to SSRS. Power BI Licensing will add additional cost and so is postponed. Work is being done to provide local agency management staff access to more WIC reports on the DPH Reporting Portal.
5.3 Develop a new MIS equipment obsolescence plan.	Continuing equipment refresh every three to four years.	Current IT infrastructure is a client/server environment on windows platform using technical control is as follows: Routers are managed by DAS Bureau of Enterprise Systems and Technology, while switches, servers, desktops, laptops, tablets and printers are managed by DPH/WIC IT, providing the necessary security, constant monitoring of database and network, equipment	Age and maintenance.	WIC IT Staff	CT WIC IT staff have been working on upgrading to new laptops and new switches have been installed at all local agency sites.

		refreshment and maintenance.			
5.4 Increase staff knowledge and utilization of current Programming languages, tools and techniques.	Staff have access to training library Training on new version of Focus reports to leverage legacy reports with new dashboard technology.	Staff have access to library as needed.	Staff are utilizing training library.	WIC IT Staff	BITS is conducting some training for Power Apps and Power BI and LinkedIn trainings are available for all state staff now.
5.5 Implement new technologies to enhance productivity and system security.	Implement Office 365 for Local Agencies to access state resources.	VPN connection to some resources, no access for others.	Local agencies able to access applicable state resources and share files via Teams.	WIC IT Staff	All staff have now access to Office 365, that also provides self-service for password resets and file sharing with state staff.
5.6 Implementation planning for next EBT contract	Complete state contract for EBT services. Convert from existing vendor to new vendor.	Waiting on NY to vacate protest on award.	NY completes master contract, contract awarded through DAS, conversion completed.	State WIC Director WIC IT Staff FRVM Nutrition Unit	Currently in negotiations with FIS, the selected contractor based on the Northeast Coalition of States bid.
5.7 Implement next MIS support contract	The existing contract expires in 2023. Plan procurement strategy with FNS and start IAPD process.	Have approval to use NASPO contract from DAS and have concurrence from FNS.	IAPD created and statement of work published.	State WIC Director WIC IT Staff	MIS maintenance and operations statement of work submitted to USDA for formal review and "meets determination" prior to execution.
5.8 Implement participant portal	Add self service capabilities so participants can pre-enroll in WIC, upload documents, and make appointments	New CT-WIC server configuration being updated to support applicable web services, and necessary security to support 40,000 users.	Self-service portal implemented	State WIC Director WIC IT Staff	The participant portal is planned for FY2025-FY2026 pending the MIS M&O SOW execution. State WIC staff are currently developing surveys for participants and local agency staff as part of the planning phase to inform development.

Program Functional Area 6: Caseload Management/Outreach

Goal 6: Effectively reach all eligible individuals as resources allow and achieve the maximum caseload capacity to serve the greatest number of women, infants, and children.

By September 30, 2024:

Objective 6.1: Target a 1% increase over 2022 first trimester enrollment rates. (14.0%-2022)

Objective 6.2: Based on available funding, continue, or expand implementation of focused Outreach Campaign - WIC is MORE in FY 2024. CDC Health Disparity grant reporting is required.

Objective 6.3: 25% of FY 2024 Local Agency Plans will include an Outreach Plan with measurable strategies.

Objective 6.4: 100% of local agencies will review and use CT-WIC show rate tracking report to improve access to WIC services. Baseline/target for show rate is 80%.

Objective	Strategies/Activities	Baseline	Indicators	Progress
6.1 Target 1% increase over 2023 first trimester enrollment rates.	Review of local agency LAPs and show that few local agencies have implemented 1st trimester enrollment outreach strategies in FY23. As a result, the State agency developed two strategies in FY24 to improve first trimester enrollment. Local agencies are required to select one for implementation. The State agency will facilitate collaboration among Program Nutritionists in the last quarter of FY23, and first quarter of FY24, to develop training, implementation, and evaluation process for the FY24 strategy.	<u>2024 WIC Objective:</u> ≥ 40% FFY 2019 Target: ≥ 40.0% Average: 19.9% Range: 3.4% - 33.8% FFY 2020 Target: ≥ 40.0%* Average: 17.4% Range: 4.3% - 38.6% FFY 2021 Target: ≥ 40.0%* Average: 13.7% Range: 0.0% - 34.3% 2022 Target: ≥ 40.0%* Average: 14.0% Range: 0.0% - 42.1% 2023 Target: ≥ 40.0%* Average: 12.8% Range: 0.0% - 29.0% Data Sources: FFY2019-2020: CT-WIC MIS (Management Information System). *Starting with 2022 State Plan submission, we will report available complete calendar year data based on previous calendar year.	CT-WIC Process Objective Report (FY 2023) 1st trimester enrollment of pregnant women is greater than or equal to 40%. DPH/DSS exchange data at least quarterly on co-enrollment between WIC & HUSKY-A.	The State agency did not meet its target for this process objective. Some reasons for this drop in first trimester enrollment may be due to staff turnover and vacancies at the local level, coupled with an influx of enrollment requests. Looking at the 2024 calendar year data (Jan-Mar) the average is 14.9%, which is an improvement over the 2023 average. The State agency has connected with a representative from Planned Parenthood Southern New England (PPSNE) to discuss updated contact information and strategies for more streamlined WIC enrollment. Local agencies have reported some issues with

				connecting with local PPSNE clinics for outreach. The FY 2025 LAP will continue to require PPSNE outreach and/or OB/GYN outreach as a strategy for this objective. The State agency has strongly encouraged the use of prenatal certification groups to expedite services to prenatal participants. Torrington Fair Haven sites continue to have some of the highest first trimester enrollment rates in 2024 Jan-Mar at 29.5% and 29.4% respectively.
6.2 Based on available funding, continue, or expand implementation of focused Outreach Campaign - WIC is MORE in FY 2023-2024. CDC Health Disparities grant reporting is required.		Baseline data in 2022	Overall participation increases (target areas = Hartford, Bridgeport, Norwich/New London, Meriden/Middletown)	Additional CDC funding was secured to continue the WIC is More outreach campaign in FY24. The continuation of the campaign ran from January to May 31, 2024. Statewide we have seen a 3.6% increase in participation rates from FY23 to FY24. Specifically in the campaign target areas, we have followed participation trends closely since the campaign began. Overall, there has been an increase in participation with a range from 0.7-15.8%. For one of the rural areas (Middlesex County) there has been a

				5.8% decline in participation from FY23 to FY24 based on closeout data.
6.3 25% of FY 2024 Local Agency Plans will include an Outreach Plan with measurable strategies.	50% of FY24 LAP's will include an evaluation of the prior year's outreach activities. Provide review for Program Nutritionists and Program Coordinators about developing targeted, measurable strategies, using updated Outreach Materials. During technical assistance visits, Local agency liaisons will discuss the development of a local Outreach Plan and measurable strategies.		LAP submission and MER Program Operations questions on Outreach.	Local agencies continue to struggle with evaluation of outreach strategies. All local agencies included outreach to either PPSNE or OB/GYN offices as part of the first trimester enrollment strategy. See Objective 6.1. 22% of local agencies included Outreach Plans with measurable strategies.
6.4 100% of local agencies will review and use CT-WIC show rate tracking report to improve access to WIC services.	During monitoring ensure local agencies are implementing proven strategies to improve show rate including • 100% of local agencies monitor show rate weekly. • Retrieve and Utilize One Call Now (OCN) report to manage clinic schedules and show rates.	Develop a realistic baseline using pre-pandemic and pandemic rates.	Review and verification of local agency process for tracking, analyzing, and implementing effective strategies to improve show rates.	The CT-WIC show rate reports for the reporting period 1/1/2023 – 12/31/2023 is 80-89% and for the period of 1/1/2024-6/25/2024 the rate is 84-90% Local agencies monitor their show rate weekly for schedule management. In several local agencies, staff are tasked with calling all participants/families assigned to them and who missed their same day appointment. The appointment may be completed later that day

	Based on FY23 MER results, the Program Operations Monitor will highlight at least 2 best practices for improving show rates at a Statewide meeting or other appropriate venue in FY24. Determine if feasible to add a show rate indicator as a LAP statewide process objective for FY25.			or rescheduled. MER observations reveal that local agencies use participant email addresses to send/receive appointment documents. Some local agencies have expanded appointment availability by offering virtual and or online appointment methods, which may be highlighted as best practices during the September Statewide meeting. Until more clarity on the no show/show rate is provided, a process objective will not be created.
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Program Functional Area 7: Coordination of Services

Goal 7: Strengthen coordination of information and resources with other Connecticut State Agencies to respond to the needs of the WIC clients.

By September 30, 2024:

Objective: 7.1 Maintain active coordination with at least 80% of identified key partners in 2024.

Objective: 7.2 90% of reviewed charts/clinic observations during monitoring visits have appropriate mandated and targeted referrals.

Objective	Strategies/Activities	Baseline	Indicators	Progress
7.1 Maintain coordination with at least 80% of identified key partners.	Continue to actively participate in State level MCH Task Forces, Head Start, Oral Health, Lead Prevention and IZ programs, HUSKY (Medicaid Managed Care), Food Policy Council (FPC), DSS and OEC. Continue to coordinate with DPH Food Protection Program, Consumer Protection Agency, and DPH Environmental Epidemiology. Facilitate emerging collaborations and/or executed MOUs with the following: • DPH led-Maternal Mortality and Morbidity Review Committee (MMRC) interagency workgroup. • Pregnancy Risk Assessment and Monitoring System (PRAMS) collaboration <i>Internal MOU executed.</i> • CT Dental Health Partnership (DHP)-<i>MOU draft in progress.</i> • Substance Exposed Pregnancy Initiative or SEPI-CT and DMHAS and DCF Policy Academy <i>Collaboration ongoing.</i> • Office of Early Childhood and Home Visiting <i>Collaboration ongoing.</i> • CT Coalition Against Domestic Violence <i>Collaboration ongoing.</i>	Letters of agreement or MOUs with Medicaid Managed Care (HUSKY), Immunization, SNAP-Ed, Department of Children and Families (DCF), and TANF. Revise as needed policy and procedures on formula safety and recall. Use existing Immunization Program MOU as a template for MOU or PSA with other programs as needed.	Improvement of service delivery to mutual clients. Ensure safety of food/formula provided to WIC participants. Current recommendations or guidelines relevant to WIC participants are sent to local WIC agencies.	The State agency maintains its engagement in many inter-agency workgroups. The bulk of the State agency efforts in FY 2024 included improving the process for dually enrolled participants in obtaining special formula through HUSKY and the development of a HUSKY/Medicaid & WIC Special Formula training for health care providers based on feedback from local agencies. WIC continues to have active MOUs with the Immunization (CT-WIZ) and PRAMS programs. Two MOUs, one between CT-DPH/WIC, DSS and CT-DHP and another between CT-DPH and OEC Home Visiting are in process (one awaiting final DSS review and the other, has been forwarded through CT-DPH for review and approval). In the absence of an MOU with OEC, the State agency

Objective	Strategies/Activities	Baseline	Indicators	Progress
	- MCH Block Grant <i>Provide data and narrative for annual reports.</i> Continue to support WIC & Head Start Better Together Collaboration at 9 local agencies in FY24 via NSA funds and existing contracts. - Sustaining State level activities include Liaison visits, bi-annual survey, and inclusion of Better Together objectives in LAP Outreach Plans. - Sustaining local level activities include co-location, collaborative nutrition and outreach, monthly meetings and As resources allow, continue SNAP-Ed/WIC Program Collaboration to complement WIC nutrition education efforts. Coordinate with SNAP-Ed as needed with local agency workshops that include taste testing and cooking demonstrations at selected sites. Incorporate routine use of SNAP-Ed recipes in WIC education/resources used. Continue participation in both the Women and Children's Health Committee and Connecticut Perinatal Quality Collaborative (CPQC). Work with both groups to better coordinate hospital and community messaging about breastfeeding to high-risk populations.	Executed updated MOU with OEC, CT-Head Start Association and 9 local grantees.	SNAP-Ed evaluations and feedback from SNAP-Ed/local agencies/students SNAP-Ed Recipes utilized at local WIC agencies. Record of Women and Children's Committee and CPQC meetings.	continues to work collaboratively (monthly check-ins) with OEC to pilot a centralized process for WIC and home visiting referrals in the Bristol/New Britain area. In FY 2024, local four new Head Start program signatories were added to Head Start data sharing MOU. A joint letter was sent to WIC and Head start Programs about reviving the WIC & Head Start Better Together Collaboration post pandemic. In FY 2025, State-level engagement is planned for those areas with new Head Start signatories. See attached summary of SNAP-Ed/WIC activities. The State agency did not participate in CPQC this year. Will remove this strategy in FY 2026.

Objective	Strategies/Activities	Baseline	Indicators	Progress
	Follow-up on implementation of Perinatal Mood and Anxiety Disorder (PMAD) self-directed online training (CT-TRAIN). Continue collaboration with UConn's Jennifer Vendetti, for technical assistance and training. Update post-tests to Microsoft Forms. Perform QA on data tables, then, implement and monitor PMAD reports in 2024. Maintain partnership with Doug Edwards from Real Dads Forever and other Fatherhood Advocates throughout the state. Doug Edwards will continue to provide training and serve as a support to the CT WIC Program in ensuring WIC services are family centered and encourage participation from dads and family men. Local agencies will build relationships with local fatherhood advocates to find site		Completion of PMAD training on CT-TRAIN and evaluation for new Staff. Ongoing implementation of WIC PMAD Screening Protocol. Improved engagement with dads and family men at local WIC offices. Local agencies will have identified father advocates in their service area to engage with for referrals and training/engagement opportunities.	WIC Numbered Memorandum 24-011 was transmitted with relevant updates to the PMAD Protocols and Resources. The State agency through a local contractor with federal SAMHSA funding held 2-virtual 2-hour trainings on 6/24/24- <i>Adult Mental Health Response</i> for WIC staff to improve confidence in conducting PMAD screening. Post-tests for PMAD TRAIN-CT modules updated to Microsoft Forms and await posting through DPH Communications. The functionality of Microsoft Forms for reporting/analysis of data trends in not ideal and NSU continues to request access to Survey Monkey for better analysis. On 12/15/23 Doug Edwards provided training on <i>'Partnering with Family Men: Why and How'</i> for all local agency WIC staff. Evaluation results show that there was a 50% increase in level of understanding on the importance of partnering with dads/family men. In addition, 78% said that they gained new techniques, skills and approaches they can apply to the jobs.

Objective	Strategies/Activities	Baseline	Indicators	Progress
	specific strategies in engaging more with dads/family men.			
7.2 90% of reviewed charts/observations during monitoring visits have appropriate mandated and targeted referrals.	Through State MER observations and chart audits 90% of local agency staff will document appropriate referrals per revised referral codes and referral policy and procedures. During technical assistance visits, Local agency liaisons will discuss the development of measurable strategies to improve provision and documentation of referrals addressing: ▪ Improved documentation on provision of referrals & follow-up. ▪ Improved consistency of use of referral codes. ▪ LA's develop internal processes for tracking referrals (providing and following up). ▪ Reduction in review findings related to referrals.	N/A	Improved local level coordination with staff regarding referrals. All local agencies will utilize a Local Community Resource Guide on a regular basis. Reduction in review findings related to referrals.	95% of charts audited had appropriate documentation of mandated referrals. Provision of targeted referrals was evident in local agencies, however one local agency failed to document provision of referrals to participants who indicated substance use. All local agencies have a LA resource guide and were observed providing the LA resource to participants. In addition, there is evidence that the LA resource is provided in documentation.

Program Functional Area 8: Civil Rights

Goal 8: Establish and administer policies and procedures that ensure participant-oriented, customer friendly service environment for all applicants and participants and comply with Racial/Ethnic Federal mandate to ensure uniformity and comparability in the collection and use of data as required by OMB standards.

By September 30, 2024:

Objective: 8.1 Verify 100% of local agencies comply with use of revised nondiscrimination statement requirements and OMB racial/ethnic data collection standards.

Objective: 8.2 Conduct annual civil rights training for state and local agency staff.

Objective	Strategies/Activities	Baseline	Indicators	Progress
8.1 Verify 100% of local agencies comply with use of non-discrimination statement requirements and OMB racial/ethnic data collection standards.	During monitoring, request copies of LA developed brochures, handbooks, and/or other publications, webpages or social media accounts and review for proper usage of the nondiscrimination statement. Monitor to verify that Racial/Ethnic Data Collection procedures are followed at local agencies during FY24 reviews.	Ongoing	Each brochure and handout and webpage will contain the current USDA non-discrimination statement. Regulatory compliance as evidenced in monitoring reports.	One of the three local agencies reviewed to date failed to ensure the current NDS is included on all publications. Staff in the three local agencies reviewed follow the proper procedure for collection of Race/Ethnic data.
8.2 Conduct annual civil rights training for local agency staff.	Annually, update and train all State and local staff on revised nondiscrimination complaint procedures and forms.		The annual interactive Civil Rights applied session is incorporated into the annual Statewide Meeting.	Annual Civil Rights Training was presented by NERO/USDA on 9/27/2023 or 9/28/2023. All State and local agency staff attended or reviewed the recording.

Program Functional Area 9: Certification & Eligibility

Goal 9: Improve and maintain program integrity in the areas of certification and eligibility.

By September 30, 2024:

Objective: 9.1 Investigate during MER (Nutrition Monitor) and State audits (Nutrition Consultant Formula Lead (NCFL)), local agency compliance with State WIC Special Formula and Eligible Nutritional policies.

Objective: 9.2 Monitor local agency implementation of updated risk criteria.

Objective	Strategies/Activities	Baseline	Indicators	Progress
9.1 Investigate during MER (Nutrition Monitor) and State audits (NCFL), local agency compliance with State WIC Special Formula and Eligible Nutritional policies.	For local agencies with MER scheduled, the Nutrition Monitor will use the CT-WIC Program Special Formula Review Form to determine if 80% of agencies reviewed comply with special formula procedures. ▪ Select 5 (per permanent site) participant records with WIC Medical Documentation forms to determine frequency of insufficient "medical rationale" and if process was followed for HUSKY issuance of special formula. ▪ Based on monitoring results, determine areas for improvement in staff training and clarify WIC formula policies and procedures and provide technical assistance as needed.	Established in 2018	Local agencies will demonstrate proficiency with: Following policy and process in assisting participants in ordering of special formula received through WIC. Following policy and process for assisting participants dually enrolled in HUSKY/Medicaid and WIC in obtaining special formula. Local agency report of improved knowledge base and comfort-level in interactions with HCP's.	In FY24, three agencies reviewed had special formula issuance findings. Most findings were related to missing documentation, inadequate follow-up tolerance or failure to assign risk factors/medical conditions identified on the medical documentation form. For one local agency there is no medical documentation form on file for participants who received special formula through WIC. 58% of completed WIC Medical Documentation forms were up to date and included information regarding HUSKY/Medicaid coverage. 80% of MER charts reviewed showed the CT-WIC Risk screen was updated based on medical conditions identified on medical documentation forms. The NCFL completed 21 chart reviews as required. The results show 84% Medical Documentation Forms Up to Date, 77% Medical Documentation Forms

Objective	Strategies/Activities	Baseline	Indicators	Progress
	The NCFL will use the CT-WIC Program Special Formula Review Form to conduct a minimum of 5 chart audits per local agency, for those local agencies conducting an Off-Year Self Evaluation. per local agency. Approximately 20 charts per year will be evaluated by the NCFL. See indicators column, for performance standards for WIC Special Formula and Eligible Nutritionals' issuance as well as HUSKY/Medicaid issuance. Based on results, local agency liaisons will discuss areas for improvement and address questions or concerns during local agency TA visits. Outstanding formula issues identified will be topic for discussion during training sessions at local agency staff meetings, Program Nutritionists and/or statewide meetings. Confirm baseline for compliance with special formula policies and practices for participants dually enrolled in HUSKY/Medicaid and WIC. (HUSKY as first payer). Determine useful tracking reports.		100% Medical Documentation Form Up to Date 70% Medical Documentation Form accurately completed by MD/HCP 90% Medical Documentation correlates with HUSKY and/or WIC prescription issuance on Family Benefit List (FBL) 80% Appropriate Growth Assessment/evaluation documented. 50% of verbal orders for medical documentation are obtained within 24 hours (WIC only). 25% of Nutritionists follow-up with MD/HCP when indicated. 60% CT-WIC Risk Screen updated based on new information.	accurately completed by MD/HCP, 73% Medical Documentation correlates with HUSKY and/or WIC prescription issuance on Family Benefit List (FBL), 25% Appropriate Growth Assessment/evaluation documented, and 41% CT-WIC Risk Screen updated based on new information. Some of the above indicators show progress from last year. However, local agency staff need improvement in Care Plan documentation and consistency. Further, more focus is needed on confirming with HCP when HUSKY/Medicaid has covered special formula. Only 38% of charts showed that staff documented in CT-WIC when HUSKY/Medicaid covered special formula. The NCFL made some modifications to chart audit indicators in 2024 based on 2023 results. This process will continue to evolve.

Objective	Strategies/Activities	Baseline	Indicators	Progress
	In the first quarter of FY 2024, NCFL will follow-up with the vendor unit on the progress of the updated data entry process for WIC Special Formulas and Eligible Nutritionals. At a minimum, review bi-annually. Update list in 1st quarter of fiscal year. Review CT-WIC quarterly reports of Issuance and Redemption of WIC Special Formulas and Eligible Nutritionals. Liaisons may choose to review during technical assistance visits as needed. Provide at least one (1) in-service training to stakeholders about changes to WIC special formula policies (HUSKY primary payor) and procedures by September 30, 2024. Assist local agencies in providing in-service presentations as appropriate.			The NCFL and FRVM unit have an ongoing process for entering new WIC Special Formulas into CT-WIC Nutrition Module in a timely fashion. The primary responsibility has been with the NCFL since 2022 during the national formula crisis. At the April Program Nutritionists' meeting, the NCFL provided an update on the above chart audit results. The State agency will update its Special Formula Review Checklist to reflect changes to State agency QA, it will also provide local agencies with its Excel spreadsheet to track results in FY 2025. See Functional Area 7: Coordination of Services for details State agency efforts to provide training and raise awareness of the new HUSKY/Medicaid and WIC Formula process. The State agency also developed a presentation for local agencies to share with their staff on the special formula process changes.

Objective	Strategies/Activities	Baseline	Indicators	Progress
9.2 Monitor implementation of updated Nutrition Risk Criteria.	By the end of first quarter, review new or updated WIC nutrition risk criteria develop training plan and submit IT request to perform needed modifications to CT-WIC. Provide update to Program Nutritionists by 2nd quarter, on results of post-test for WIC Nutrition Risk Criteria and WIC Nutrition Risk Assessment training. (Self-paced module). Train local agency staff on new or revised criteria at Statewide meetings or using a self-paced module. During monitoring determine if local staff accurately identify and assign new or revised risks. Through State MER observations and chart audits 80% of local agency staff will conduct PMAD screening per protocol and document appropriately.	N/A	IT report submitted. Training scheduled for local agencies. Monitoring indicates local agency staff are correctly assigning risks.	The State agency made all updates for new and revised risk criteria to be implemented 10/1/2024, during FY 2023-staff training, changes to tools and forms. The Nutrition Risk updates requiring MIS changes will be included in CT-WIC Release 4.2, scheduled for August 2024. The intended implementation date 1st quarter of FY 2024 was pushed back due to work on the CT-WIC finance module. An update will be sent to local agencies when CT-WIC changes are finalized. One of the three agencies reviewed in FY24 had findings related to staff accurately identifying and/or assigning risks. Two of the three agencies reviewed had findings related to either documentation (i.e., scanning or data entry in CT-WIC) on the PMAD screen or completing the PMAD screens at the designated timeframes. 60% of local agency staff reviewed, conducted PMAD screening per protocol and of those, 75% documented appropriately based on chart audit review.

Program Functional Area 10: Monitoring & QA

Goal 10: Conduct on-site programmatic management and nutrition services reviews of local agencies to ensure compliance with Federal and State Regulations.

By September 30, 2024:

Objective: 10.1 Monitor five (5) service regions including satellites.

Objective: 10.2 Evaluate applications of VENA principles in local operations and identify training and technical assistance needs.

Objective	Strategies/Activities	Baseline	Indicators	Staff Assigned
10.1 Monitor five (5) service regions including satellites.	By end of 1st quarter, develop FY24 monitoring schedule. - Conduct monitoring visits, schedule exit conference within two weeks of completion of fieldwork, and complete exit conference within 30 calendar days. - Prepare program review reports to local agencies within thirty (30) calendar days of the program exit conference. - Respond to local agency CAP within 30 days. Two weeks as best practice. - Synthesize common review findings & responses to CAP in both nutrition services and program operations to update FY24 Goals and Objectives, training, and technical assistance plans. - During routine monitoring, collect data on satellite site operations to determine effectiveness: Location of satellite sites, # of participants served, and Hours of operation	FY21 LA monitoring schedule (See Objective 10.3)	100% of scheduled monitoring visits and reports completed by Sept 2024. 80% of agencies will have their exit conference scheduled within two weeks of completion of onsite visits.	The monitoring schedule was developed at the beginning of the 1st quarter. Five agencies were scheduled for routine monitoring in 2024. In addition, one agency is scheduled for a follow up visit based on ME findings from 2023. Three agencies have been reviewed to date. One agency will not be reviewed due to contract termination. The last routine review and follow up visit will be conducted by September 2024. Common findings and CAP responses will be used to update FY25 Goals and Objectives, and training and technical assistance plans.

10.2 Evaluate applications of VENA principles in local operations and identify training and technical assistance needs.	Monitor local agency staff progress in three (3) VENA competency areas to establish individual baselines for self-improvement. Highlight local agency best practices once a year as appropriate. Utilize Program Nutritionist meetings to address local agency training and technical assistance needs. ▪ Update web page as needed. ▪ Upload to CT-TRAIN and implement modules on Nutrition Risk for CPAs/Nutritionists. ▪ Determine next steps to offer hybrid (virtual and in person) bi-annual New Staff Orientation using lessons learned from and technology acquired during the pandemic.		Ongoing process/tool evaluation and feedback from local agencies.	Two of the three agencies reviewed to date were observed effectively utilizing VENA skills during appointments observed. One local agency needed improvement in risk assignment and nutrition education documentation. One local agency reviewed struggles with all areas of VENA including rapport building, critical thinking, and positive health outcomes. The local agency is required to conduct training for all areas of VENA as a part of the CAP. At the WIC 50th Celebration in September 2024 local agencies will present their best practices. The State agency developed and presented several trainings to address trending MER findings: Nutrition Risk Assessment and Documentation and Data Quality. The State agency has not been able to have training posted to TRAIN-CT in a timely fashion due to internal DPH Communications resource issues. The State agency has offered two hybrid new Staff Orientations in FY 2024 (December/June) as well as a virtual new Staff Orientation for Waterbury in September 2023 to assist the Optimus management in onboard Waterbury staff as the lead contractor on 10/1/2024.
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Program Functional Area 11: Fiscal Management

Goal 11: Maximize the utilization of WIC food funds.

By September 30, 2024

Objective: 11.1 Keep food dollars usage stable at 90%.

Objective: 11.2 Use economic and financial trend data to manage resources and improve program quality more effectively.

Objective: 11.3 Ensure drawdowns are processed against the correct accounts.

Objective	Strategies/Activities	Baseline	Indicators	Staff Assigned	Progress
11.1 Stable usage of food funds at 90%.	- Track and monitor monthly food expenditures. - Meet with program coordinators. - Work with WIC Director and Epidemiologist to monitor food costs using the current CPI cost indicators.	Budget/expenditure reconciliation.	100% of food dollars are accounted for.	State WIC Director Fiscal Unit Epidemiologist	The objective has been met and we will continue to use the strategies/activities in our ongoing procedures.
11.2 Use economic and financial trend data to manage resources and improve program quality more effectively.	Utilize financial data in trend analysis.	N/A	Utilize financial trend data to drive program decisions.	State WIC Director Fiscal Unit Epidemiologist	The objective has been met and we will continue to use the strategies/activities in our ongoing procedures.
11.3 Ensure drawdowns are processed against the correct accounts.	Create process to have two fiscal staff complete and review draw information to ensure accuracy.	N/A	Draw downs are correctly processed.	Fiscal Unit	The objective has been met and we will continue to use the strategies/activities in our ongoing procedures.

Program Functional Area 12: Data Quality, Analysis & Reporting

Goal 12: Strengthen data outputs for improved program planning, monitoring, evaluation, and administration.

Objective 12.1: Improve access to, and the utility and application of, WIC Program data:

- a. Build on current reports to provide enhanced, accurate and more accessible, data resources;
- b. Expand research/data analysis and reporting initiatives; and,
- c. Provide support in meeting other Program-related data needs.

Objective 12.2: Contribute data inputs to help maximize strategic program coverage and effectiveness:

- a. Strengthen appropriate access to and delivery of program services; and,
- b. Ensure adequate access to vendor services and vendor capacity to meet demand.

Objective	Strategies/Activities	Baseline	Indicators	Progress
12.1 Improve access to, and the utility and application of, WIC Program data: a. Build on current reports to provide enhanced, more accessible data resources.	Prepare summary data tables & graphs to illustrate trends, and maps to compare the distribution of selected variables and other Program resources (e.g., participants, WIC sites, authorized vendors). Post results or otherwise share selected data tables, graphs, reports, maps, and other data presentations as appropriate, including: • WIC participation and key demographics; • Process and outcome objectives; • Benefit issuance & redemption; and, • Authorized vendors; plus, • Current national WIC data and state population figures. Coordinate with DPH attorney prior to sharing any personally identifiable information outside the WIC program (e.g., with DPH/Vital Records, Local Health Districts), to ensure state and federal confidentiality regulations are properly met; censor all public-facing data, as appropriate.	Monthly Reports Quarterly Outcome Reports Annual Summary Reports	Enhanced analysis & data presentations meet USDA, state & local WIC agency data needs. Data tables, graphs & maps facilitate the comparison of participant characteristics, outcomes, etc. Data reports are posted to the shared Q drive for state & local agency staff; public-facing data posted to program website and DPH Dashboard are censored and periodically updated, as appropriate. Improved data access results in less staff time being invested in responding to routine data requests, while protecting file security & participant confidentiality.	A solution to making WIC reports available to local agency contractors continues to be explored. These reports are now available to State Staff via Teams WIC Reports. USDA, State & Local data needs continue to be met
b. Expand research/data analysis and reporting initiatives.	Select one program outcome to analyze in relation to potential risk/contributing factors (e.g., low birth weight: compare preterm/full-term birth outcomes in relation to mothers' pre-pregnancy weight, weight gain during pregnancy, substance use behaviors, initiation of prenatal care, etc.).	Prior DPH, WIC and other related studies DSS Medicaid/HUSKY A dataset	More in-depth analyses help inform program decisions in support of key interventions, with a focus on improved outcomes, and increased outreach, recruitment & retention.	Pending analysis of one program outcome in relation to potential risk/contributing factors. The focus for 2024 has been the complete re-design of CT WIC's six

	• Identify and track risk factors contributing to poor birth outcomes. • Evaluate associations between WIC participation and risk factors for poor birth outcomes. • New addendum to LBW outcome report will track prematurity data (current report excludes multiple and preterm births). Support all efforts to procure DSS/Medicaid dataset, in order to: • Determine co-enrollment in WIC/Medicaid: • Use Medicaid data to identify and provide outreach to those eligible but not enrolled in WIC; map addresses to identify pockets of those not enrolled, enabling more targeted outreach efforts. • Map selected health, demographic and socio-economic Census variables at the local level; compare results with current program coverage.	U.S. Census Bureau data	Newly redesigned Outcome reports used to develop the methodology to: • Perform crosstab analyses of Outcomes to determine impact on results by, for example, race and ethnicity (e.g., breastfeeding initiation & duration); • Explore the impact of risk factors on Outcomes; and, • Map results. Continued support of all efforts to procure DSS/ Medicaid dataset (unsuccessful to date).	Outcome Reports, to improve reliability, readability, and end user functionality. Still no significant progress on efforts to procure DSS/Medicaid dataset.
c. Provide support in meeting other Program-related data and reporting needs.	Respond to internal & external WIC data requests (e.g., DPH, Local Health Districts, FOI requests, universities, non-profits, <i>ad hoc</i> requests, etc.). Identify/develop relevant reference & training resources: • Draft presentations for WIC Director, Nutrition staff, Vendor Unit, etc., when requested. • Prepare reports for use by state staff in local agency monitoring visits (e.g., staff-to-participant ratio analysis by race/ethnicity; benefit redemption by Permanent Site, etc.). • Develop presentations for WIC staff; present on data-related topics as needed at statewide meetings, coordinators meetings, trainings. • Provide analysis, reporting and/or technical assistance as requested (e.g., adjust annual WIC outcome targets, calculate caseload, design surveys and new reporting forms, etc.).	CT-WIC data reports Ongoing collaboration	Timely response to internal and external data requests. Data reports, surveys and presentations developed and/or technical assistance provided. Committees successfully complete assigned tasks.	Ongoing.

	• Contribute to preparing the Annual State Plan and yearly Operational Adjustment proposal; present midterm, and final reports. • Collaborate in DPH initiatives that benefit the State's MICH population (e.g., MCH Block Grant, PRAMS Steering Committee, RFPs, Publication Review Committees, etc.).			
12.2. Contribute data inputs to help maximize strategic program coverage & effectiveness: a. Strengthen appropriate access to and delivery of program services.	Monitor Program services to help inform program planning and implementation efforts: • Identify service gaps, priorities & opportunities. • Track program outcomes, evaluate changes. Use GIS mapping to help improve resource allocation (e.g., location of clinic sites, authorized vendors). Target resources to improve outcomes for those at highest risk. Identify specific populations to target for outreach promotional efforts and program services, where needed. Develop new reports (e.g., identification of Risk Factors, Referrals/Counter-Referrals by local agency, and permanent WIC site, etc.).	Current program services & resources	Decisions to increase/decrease or relocate program services and resources are based on objective inputs (e.g., trend analysis, percentage change over "x" period of time, etc.). Risk factors and other variables associated with a specific outcome or result(s) are identified, providing enhanced criteria for targeting program interventions.	Ongoing.
b. Ensure adequate access to vendor services, and vendor capacity to meet participant demand.	Coordinate with the Vendor Management Unit to strengthen data collection, analysis and reporting functions, especially for vendor monitoring and TIP Report preparation. Continue to identify priority data needs; perform needed analyses and build standardized queries and presentation formats to meet those needs.	Vendor Unit data Authorized Vendor lists WIC Participation data	Vendor locations, services and resources meet participant and program needs. Analytical maps determine geographic access and store capacity to meet demand of WIC participants produced prior to adding or removing a vendor from the CT WIC authorized vendors list.	Ongoing, as requested.

FFY 2025

GOALS AND OBJECTIVES

Program Functional Area 1: Management and Organization

Goal 1: Ensure program integrity, cost-effectiveness, quality of services and accountability in the delivery of nutrition services and the provision of food benefits.

By September 30, 2025

Objective 1.1: Convene monthly meetings (in-person or virtual) for local agency coordinators to ensure on-going communication and feedback loop.

Objective 1.2: Actively participate in statewide efforts to increase collaboration, referral, and data sharing, including continued work on myCT, the state's health and human service portal and meetings with the CT Department of Social Services, Department of Agriculture, and State Department of Education.

Objective 1.3: Convene at least one local agency management staff meeting, focused on emerging WIC topics, best practices, data management, and/or leadership.

Objective 1.4: a) Update DPH and WIC Continuity of Operations Plan (COOP) to include recommended USDA components and ensure alignment.

b) Finalize internal operating procedures and provide state and local agency training.

Objective 1.5: Create a participant advisory council to support health equity and provide input on program modernization and improvement efforts.

Objective 1.6: Conduct a cost-effectiveness study of local agency contractors.

Objective	Strategies/Activities	Baseline	Indicators	Staff Assigned
1.1 Convene monthly meetings (in-person or virtual) for local agency coordinators to ensure on-going communication and feedback loop by 9/30/25.	Schedule and facilitate at least 10 meetings with Program Coordinators by 9/30/25 to: • Review policies and procedures. • Discuss funding and staffing issues. • Review CT-WIC data availability and reports. • Other topics as determined.	N/A	Summary of meetings. Improved compliance with policies and procedures. Increased sharing of strategies regarding resource utilization and staff retention. Improved use of data and discussions in LAP.	WIC Director State Nutrition Coordinator

1.2 Actively participate in statewide efforts to increase collaboration, referral, and data sharing by 9/30/25, including continued work on myCT, the state's health and human service portal and attend meetings with the CT Department of Social Services, Department of Agriculture, and State Department of Education to coordinate service delivery of federal programs.	• Attend meetings through 9/30/25. • Provide clarification and information regarding WIC Program Regulations and policies. • Create a plan for developing an MOU with state agency partners to data share by 1/1/25.	N/A	WIC Program successfully integrated into statewide efforts. Plan for data sharing MOU completed.	WIC Director State Nutrition Coordinator Nutrition Consultant
1.3 Convene at least one local agency management staff meeting, focused on emerging WIC topics, best practices, data management, and/or leadership by 5/31/25.	• Identify date, time, and location of meeting. • Select topics for inclusion, plan agenda, identify meeting objectives.	N/A	Meeting completed and summary created. Meeting objectives met.	WIC Director State Nutrition Coordinator
1.4 a) Update DPH and WIC COOP to include recommended USDA components and ensure alignment by 8/15/25. b) Finalize the State Agency Internal Operating Procedures Plan (Disaster Plan) by 8/15/25 and provide state and local agency training by 9/30/25.	• Work with DPH's Public Health Preparedness and Local Health Section to review draft WIC COOP components and identify gaps/areas of improvement. • Update as appropriate. • Update Disaster Plan as needed to address gaps and meet new requirements. • Provide state and local agency training.	N/A	DPH/WIC COOP component finalized and integrated in DPH COOP. State Agency Internal Operating Procedures finalized. State and local agency training completed.	WIC Director State Nutrition Coordinator FRVM Supervisor

1.5 Create a participant advisory council to support health equity and provide input on program modernization and improvement efforts by 9/30/25.	• Work with local agency staff to assist with recruiting participant members throughout the state. • Convene at least two meetings, one by 6/30/25 and one by 9/30/25. • Plan topics for inclusion, develop agendas, and identify meeting objectives	N/A	Participant advisory council formed. Meetings completed, summaries created, and objectives met.	WIC Director State Nutrition Coordinator FRVM Supervisor
1.6 Conduct a cost-effectiveness study of local agency contractors by 7/31/25.	• Assess existing staff-to-participant ratios, location and distribution of participants, and distribution of potentially eligible participants, etc. by 3/31/25. • Compile a report on assessment and next steps to inform future local agency contract cycles by 7/31/25.	N/A	Report completed.	WIC Director State Nutrition Coordinator

Program Functional Area 2: Nutrition Services, Breastfeeding Promotion and Support*

Goal 2: Improve the nutritional and overall health of WIC families in Connecticut.

By September 30, 2025,

Objective 2.1: At least 35% of pregnant women participating in the WIC Program for a minimum of 6 months gain appropriate weight.

Objective 2.2: The incidence of low birth weight (LBW) among infants whose mothers were on the WIC Program for at least six months during pregnancy does not exceed 4%.

Objective 2.3: The prevalence of anemia among children enrolled in the WIC Program for at least one year does not exceed 7.5%.

Objective 2.4a: The prevalence rate of BMI $\geq$ 85th percentile to < 95th percentile for children 2-5 years does not exceed 15%.

2.4b: The prevalence rate of BMI $\geq$ 95th percentile for children 2-5 years of age does not exceed 10%.

Objective 2.5: At least 80% of infants enrolled in the WIC Program (have mothers who) initiate breastfeeding.

Objective 2.6: Define and develop breastfeeding duration report and target.

Objective	Strategies/Activities	Data	Indicators	Staff Assigned
2.1 At least 35% of pregnant women participating in the WIC Program for a minimum of 6 months gain appropriate weight.	Tell me More About You (TMM) strategies for prenatal participants to improve appropriate MWG were continued for the FY 2025 LAP. It will be determined if a baseline can be established with FY 2025 LAP results. Local agency liaisons will perform TMM QA on prenatal charts in FY 2025. A total of 20 chart audits (sample to include prenatal participants the 4 pre-pregnancy BMI classifications) to determine completeness of nutrition education documentation.	<u>2025 WIC Objective:</u> $\geq$ 35% <u>FFY 2019 Target:</u> $\geq$ 35.0% Average: 27.7% Range: 13.5% - 46.3% <u>FFY 2020 Target:</u> $\geq$ 35.0%* Average: 26.5% Range: 7.1%-40.2% <u>FFY 2021 Target:</u> $\geq$ 35% Average 27.0% Range: 6.9-74.3% <u>2022 Target:</u> $\geq$ 35%* Average 27.5% Range: 4.9-42.9% <u>2023 Target:</u> $\geq$ 35%* Average 28.3% Range: 11.6% - 47.5% <u>2024 data pending.</u> <u>Data Sources:</u> FFY2019-2021: CT-WIC MIS (Management Information System).	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports. MER chart reviews and Observations. Improvement in trend data over time for low performing agencies.	Nutrition Monitor Local Agency Liaisons Epidemiologist IT Unit Nutrition Unit

		*Beginning with 2022 State Plan submission, we will report available complete calendar year data.		
2.2 The incidence of low birth weight (LBW) among infants whose mothers were on the WIC Program for at least six months during pregnancy does not exceed 4%.	Through State MER observations and review of documentation in chart audits, 80% of local agency staff incorporate prenatal weight gain guidance concepts in counseling, provide timely referrals and offer appropriate education materials that address applicable <i>modifiable</i> risk factors: ▪ Smoking ▪ substance use (including prescription drugs) ▪ Prenatal weight gain ▪ Mental health concerns (PMAD) Investigate Local Agency Plan (LAP) community needs assessments to determine baseline of LAP's that include data and/or add context to variables that impact LBW deliveries: • teenagers • substance use • income • ethnicity • geographic location • mental health Maintain relevant State agency collaborations that impact this outcome objective. (See Functional Area X)	2025 WIC Objective: ≤ 4% FFY 2019 Target: ≤ 6.0% Average: 2.5% Range: 0.0% - 13.6% FFY 2020 Target: ≤ 6.0%* Average: 2.7% Range: 0.0% - 13.8% FFY 2021 Target: ≤ 6.0% Average: 3.1% Range: 0.0% - 9.5% 2022 Target: ≤ 4.0%* Average: 3.1% Range: 0.0% - 12.5% 2023 Target: ≤ 4.0%* Average: 4.7% Range: 0% - 17.2% 2024 data pending. FFY2019-2021: CT-WIC MIS (Management Information System). (*excludes pre-term & multiple births) *Beginning with 2022 State Plan submission, we will report available complete calendar year data.	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports. Improvement in trend data over time for low performing agencies.	Nutrition Monitor Local Agency Liaisons Epidemiologist Nutrition Unit

2.3 The prevalence of anemia among children enrolled in the WIC Program for at least one year does not exceed 7.5%.	Through State MER observations and chart audits 90% of local agency staff effectively provide education to parents to reduce risk of development of anemia including: ▪ Importance of timely blood work, ▪ Risk of lead poisoning ▪ Making appropriate referrals and follow-up. Local agency liaisons will perform anemia QA chart audits in FY24. A total of 15 chart audits (sample to include 10– 9-month IBE and IBP- (Mostly) infants; and 5- C1 to determine if anticipatory guidance and anemia prevention messages related to use of infant meat and cereals are documented at nutrition education visits.	<u>2025 WIC Objective:</u> ≤7.5% FFY 2019 Target: ≤ 7.5% Average: 8.5% Range: 3.9% - 19.0% FFY 2020 Target: ≤ 7.5% Average: 8.5% Range: 3.5% - 18.2% FFY 2021 Target: ≤ 7.5% Average: 8.8% Range: 2.4% - 17.5% 2022 Target: ≤ 7.5%* Average: 8.2% Range: 2.9% - 12.7% 2023 Target: ≤ 7.5%* Average: 8.6% Range: 2.9% - 12.7% 2024 data pending. FFY2019-2021: CT-WIC MIS (Management Information System). *Beginning with 2022 State Plan submission, we will report available complete calendar year data.	CT-WIC quarterly and annual reports. Chart audits and observations. Change in trend data over time for low performing agencies.	Nutrition Monitor Local Agency Liaisons Epidemiologist Nutrition Unit
2.4 a. The prevalence of BMI ≥ 85thile to < 95thile for children 2-5 years of age does not exceed 15%. b. The prevalence of BMI ≥ 95thile for children	TMM strategies for child participants at risk of overweight or obesity were continued for the FY 2025 LAP. It will be determined if a baseline can be established with FY 2025 LAP results. The State agency will provide a presentation on the results of the	<u>2025 WIC Objectives:</u> Overweight: ≤ 15% Obesity: ≤ 10% a. OVERWEIGHT: BMI ≥ 85thile to <95thile FFY 2019 Target: ≤ 15% Average: 15.1% Range: 10.1% - 20.1%	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports. Indicators of effective use include documentation of TMM, MI, Stages of Change and/or development of a plan.	Monitoring Staff Local Agency Liaisons Epidemiologist IT Unit Nutrition Unit

2-5 years of age does not exceed 10%.	FY24 TMM chart audits by December 2024 and identify areas for improvement in counseling and documentation for children at risk of overweight/obesity. Local agency liaisons will perform TMM QA chart audits in FY 2025. A total of 15 chart audits (sample to include child participants with at risk for or with overweight or obesity risk) to determine completeness of nutrition education documentation.	FFY 2020 Target: ≤ 15% Average: 15.1% Range: 9.1% - 20.8% FFY 2021 Target: ≤ 15% Average: 14.7% Range: 8.6% - 22.9% 2022 Target: ≤ 15%* Average: 14.4% Range: 10.0% - 18.6% 2023 Target: ≤ 15%* Average: 14.7% Range: 8.6% - 20.2% b. <u>OBESITY</u>: BMI ≥ 95thtile FFY 2019 Target: ≤ 10.0% Average: 15.3% Range: 9.3% - 22.8% FFY 2020 Target: ≤ 10.0%* Average: 15.5% Range: 7.7% - 28.3% FFY 2021 Target: ≤ 10.0%* Average: 19.1% Range: 12.5% - 28.1% 2022 Target: ≤ 10.0%* Average: 16.7% Range: 8.8% - 23.1% 2023 Target: ≤ 10.0%* Average: 15.8% Range: 10.5% - 21.2% 2024 data pending. FFY2019-2021: CT-WIC MIS (Management Information System).		
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		*Beginning with 2022 State Plan submission, we will report available complete calendar year data.		
2.5 At least 80% of infants enrolled in the WIC Program (have mothers who) initiate breastfeeding.	Through State MER observations and chart audits 50% of local agency staff will incorporate and document concepts from breastfeeding Content Sheets into prenatal education and counseling. MAP strategies for prenatal participants interested in providing human milk were continued for the FY 2025 LAP. If feasible, a baseline will be established with FY 2025 LAP results. Two of the FY 2025 monthly breastfeeding coordinators meetings will include discussions of the LAP initiation strategy and implementation best practices from local agencies. The Breastfeeding Unit will conduct 1-2 enhanced breastfeeding reviews at local agencies. (One review will include agency with peer program.) MAP strategies for prenatal participants interested in providing human milk were continued for the FY 2025 LAP. If feasible, a baseline will be established with FY 2025 LAP results.	2025 WIC Objective: ≥80% <u>FFY 2019 Target:</u> ≥ 70.0% Average: 81.6% Range: 59.9% - 94.4% <u>FFY 2020 Target:</u> ≥ 70.0%* Average: 81.3% Range: 63.9% - 93.6% <u>FFY 2021 Target:</u> ≥ 80.0%* Average 79.8% Range: 61.1% - 94.3 % 2022 Target: ≥ 80.0%* Average 79.6% Range: 62.8% - 94.9% 2023 Target: ≥ 80.0%* Average 79% Range: 58.1% - 94.6% 2024 data pending. FFY2019-2021: CT-WIC MIS (Management Information System). *Beginning with 2022 State Plan submission, we will report available complete calendar year data based on previous calendar year.	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports, Chart Audits and Observations conducted during Enhanced Breastfeeding Reviews and Peer Counseling Monitoring. Results of enhanced Breastfeeding MERs, show documentation of 80% compliance with CT Guidelines for Breastfeeding Promotion and Support.	Nutrition Monitor Local Agency Liaisons Epidemiologist IT Unit Breastfeeding Unit Epidemiologist

	Two of the FY 2025 monthly breastfeeding coordinators meetings will include discussions of the LAP initiation strategy and implementation best practices from local agencies. The Breastfeeding Unit will conduct 1-2 enhanced breastfeeding reviews at local agencies. (One review will include agency with peer program.) See 2.6.			
2.6 Define and develop a breastfeeding duration report and target.	The Epi and Breastfeeding Units will work to develop a new breastfeeding duration report. Convene at least 2 meetings prior to the end of FY 2024 to finalize reporting methodology and determine baseline. Begin internal reporting of breastfeeding duration at 6 months using new methodology in FY 2025. Prepare to finalize target for FY 2026 LAP. Implementation of BF/FU appointments were continued as a FY 2025 LAP strategy for breastfeeding duration. Local agencies will use the guidance developed by the BF Coordinators to evaluate and refine their schedules,	<u>2025 WIC Objective:</u> ≥ 60% <u>FFY 2018 Target:</u> ≥ 50% Average: 69.4% Range: 44.9% - 91.4% <u>FFY 2019 Target:</u> ≥ 50% Average: 65.2% Range: 37.5% - 90.7% <u>FFY 2020 Target:</u> ≥ 50% Average: 64.4% Range: 42.6% - 88.9% <u>FFY 2021 Target:</u> ≥ 60%* Average: 61.8% Range: 38.6% - 89.3% <u>FFY 2022 Target:</u> ≥ 60%* Average: 62.7% Range: 37.7% - 90.4% 2023/2024 data pending.	CT-WIC Quarterly & Annual Outcome, Summary & Trend Reports. Chart audits and Observations.	Breastfeeding Unit Epidemiologist

	breastfeeding support, documentation and breastfeeding rates. Based on results reported in FY25 LAP, determine training needs or adjustments to polices. Provide oversight and technical assistance to 7 WIC clinic based Breastfeeding Peer Counseling Programs and one hybrid program through on-site visits, conference calls and review of quarterly program and financial reports. Continue to monitor implementation of peer-counseling module in CT-WIC development of consistent, automated data reports for peer counseling programs.	FFY2019-2021: CT-WIC MIS (Management Information System). *Beginning with 2022 State Plan submission, we will report available complete calendar year data.	Biannual activity and monthly expenditure reports from peer counseling contractors, including # of participants enrolled, and duration rates are reviewed and approved. CT-WIC Peer counseling modules are being used appropriately in agencies with peer counseling programs.	
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±See Functional Area 12, Data Quality, Analysis and Reporting for information Epidemiology and IT specific activities related to these strategies.

Program Functional Area 3: FOOD DELIVERY AND FOOD INSTRUMENT ACCOUNTABILITY

Goal 3: To improve food delivery operations at the state and local agency level.

Objective: 3.1 Improve shopping experience for participants.

Objective: 3.2 Support of Department of Agriculture in electronic Farmers Market Nutrition Program.

Objective: 3.3 Make changes to food packages and eligible foods in response to Final Rule for Revisions in the WIC Food Packages.

Objective	Strategies/Activities	Baseline	Indicators	Staff Assigned
3.1. Improve shopping experience for participants.	Standardized checklist for all local agencies to train participants on obtaining balance inquiries, reviewing the Approved Food Guide, identifying WIC approved foods using shelf labels, using the WIC Shopper app and troubleshooting WIC transaction issues.	October 2019	The creation of a standardized training program and consistent use of it, leading to fewer complaints and phone calls from participants about food benefit usage.	FRVM staff Nutrition staff
	Streamline submissions for the Connecticut WIC Approved Product List on a quarterly basis using a standard submission spreadsheet and tool for populating the CT WIC database with new products.	October 2024	Manual entries into the current spreadsheet of submission data received via text in emails, Excel spreadsheets and Word documents	FRVM staff Nutrition staff
	On an annual basis, request that each manufacturer, distributor, broker, submitter of food items, etc. from the previous fiscal year review the current APL on the CT WIC website to confirm that their respective food products and sizes remain available for purchase in Connecticut.	October 2024	Disaster Plan	FRVM staff Nutrition staff

	Additions of approved products will be provided in a format that can be loaded into the database and deactivations of discontinued products will continue to be changed manually in the Nutrition Module of CT-WIC.	November 2024	Manual entries into the current spreadsheet of submission data received via text in emails, Excel spreadsheets and Word documents	FRVM staff Nutrition staff
	Annually after confirmation is received and products have been added or deactivated, CT WIC will review the APL to ensure that products are approved in each subcategory that allow the purchase of the maximum monthly allowance.	October 2024	Disaster Plan	FRVM staff Nutrition staff
	Collaborate with Rhode Island and West Virginia WIC Programs to explore the feasibility of online shopping with curbside pick up.	April 2024	Rhode Island submission of Center for Nutrition and Health Impact proposal for WIC Online Shopping Sub-grant Project leveraging the WICShopper App	FRVM staff
3.2 Support of Department of Agriculture (DoAg) in electronic Farmers Market Nutrition Program (eFMNP).	Provide number of eligible WIC participants to DoAg and the list of eligible participants in a data file for Nations Benefits by April 1.	April 1, 2024	Program informed on 1/24/24 that the contractor is now Nations Benefits who acquired SoliSystems	FRVM staff Nutrition staff IT staff
	Provide periodic data transfer files to update WIC participant information.	May 1, 2024	Files are uploaded to an ftp site May 1-November 15	FRVM staff Nutrition staff IT staff
	Using the SoliPortal, provide assistance to families who contact the state or local offices with questions about receiving eFMNP benefits.	June 2024	DoAg implemented eFMNP using SoliSystems Corporation in July 2023 and continues to use the SoliPortal	FRVM staff Nutrition staff

	Promote nutrition education materials and collaborate with DoAg on messaging to participants about benefit issuance and spending FMNP benefits through the 2025 season.	June 2024	Messaging begins in June on the issuance of eFMNP benefits	FRVM staff Nutrition staff
	Distribute and promote the eFMNP participant surveys.	November/December 2024	Responses to surveys accessed through WIC Shopper App, WIC website and One Call Now text messaging.	FRVM staff Nutrition staff
3.3 Make changes to food packages and eligible foods in response to Final Rule for Revisions in the WIC Food Packages.	Assessment of tasks to be completed in the food resources and vendor management areas.	Final rule published 4/18/24	Current functionality of CTWIC and processes.	FRVM staff Nutrition staff
	Changes to minimum inventory, pricing and monitoring report screens.	Final rule published 4/18/24	Current redemption percentages for the highest redeemed food items	FRVM staff Nutrition staff
	Review submissions from manufacturers and distributors on product nutrition labels, statewide availability, and retail costs.		Emails and information submitted to the State WIC Office for approval in Connecticut.	
	Amendments to the vendor agreement and update the minimum inventory requirements.	Final rule published 4/18/24	Agreements are revised and emailed to all authorized vendors.	FRVM staff
	Provide bulletin on new regulations or procedures.	Final rule published 4/18/24	Bulletins sent to all WIC authorized vendors.	FRVM staff
	Communicate necessary changes in food packages to IT.	Final rule published 4/18/24	Through the MIS contractor, Voyatek, requesting and testing system changes.	Nutrition staff
	Provide updates to local agencies regarding any new policies, educational materials, etc.	Final rule published 4/18/24	Numbered memos.	Nutrition staff
	Notify wholesale distributors of updates to food packages, new food items, and minimum inventory requirements.	Final rule published 4/18/24	Mailing list and communications through the WIC Vendor Advisory Group.	FRVM staff

	Assess percentage of category redemptions to determine any increases in redemptions as a result of the new food package revisions.	Final rule published 4/18/24	Report-Redemptions by Site and Food Category	FRVM staff
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Program Functional Area 4: Vendor Management

Goal 4: To improve communication and effectiveness in Vendor Management.

Objective: 4.1 Potential transition of EBT Processors

Objective: 4.2 Develop policies and establish business processes for a Policy and Procedure Manual.

Objective: 4.3 CTWIC Enhancements to streamline processes

Objective: 4.4 Training videos to supplement interactive vendor training

Objective: 4.5 Improve investigation services through RFP process and inventory audits.

Objective	Strategies/Activities	Baseline	Indicators	Staff Assigned
4.1 Potential transition of EBT Processors	Participate in Northeast Coalition of States (NCS) WIC Only monthly conference calls to finalize EBT Services with FIS.	Joint program calls began January 7, 2021; WIC program calls separate from SNAP began in May 2024.	Discussion of any common areas between states for contracting, timelines, and communications.	FRVM staff IT staff
	Participate optionally in NCS SNAP Only monthly conference calls for updates on FIS implementation for SNAP in NERO states.	Joint program calls began January 7, 2021; SNAP only calls separate from WIC began in April 2024.	Discussion of any common areas between states for contracting, timelines, and communications.	FRVM staff IT staff
	Contract negotiations with FIS.	September 13, 2023	Bid submission May 13, 2021; FIS notified on 6/6/22 of their tentative awardee status for the EBT procurement. CT WIC notified on 3/29/23 that NY SNAP's contract was signed by FIS and other states could begin contract discussions. CT WIC notified on 9/13/23 of the bid protest from Conduent being resolved and the execution of New York SNAP's contract.	FRVM staff IT staff Department of Administrative Services (DAS)
	DAS Draft WIC Contract and Statement of Work to USDA and FIS for review.	June 2024	DAS approval of NASPO type contract provided on June 21, 2019.	FRVM staff IT staff

	Notify WIC Vendor Advisory Council and authorized vendors of contractor selection.	Q1 of FY25	Vendor community informed on potential changes.	FRVM staff
	Finalize and execute WIC EBT contract with FIS.	Q4 of FY24	DAS approval of NASPO type contract provided on June 21, 2019.	FRVM staff IT staff
	Transition and Conversion plan for data, stand beside devices, retailer portal.	2025	Transition from Conduent to new provider, FIS	FRVM staff
	Transaction testing after conversion to the new 2022 WIC Universal MIS EBT Interface (WUMEI) between TCC/Voyatek and CDP WIC Direct systems.	Q2/Q3 of FY25	Upgrade of the WUMEI to the 2022 version.	FRVM staff
4.2 Develop policies and establish business processes for a Policy and Procedure Manual.	Document new processes in response to changes to current business practices.	March 2020	Changes to the WIC Vendor Agreement, access criteria, high risk criteria	FRVM staff
	Revise policies and establish business processes for a Policy and Procedure Manual.	Policies and procedures in writing.	Implementation of eWIC, utilization of CT-WIC, and the COVID-19 pandemic changed processes. Shared knowledge of processes.	FRVM staff
	Modify the CT WIC Manual for the Vendor Module based on changes and enhancements made.	April 2018	Vendor Module Manual updated last for version 1.9.	FRVM staff Nutrition staff
4.3 CTWIC Enhancements to streamline processes.	Enhancements to Vendor Module training screens to increase the functionality and connection between training, monitoring and competitive pricing phases with selection criteria verification.	August 2020	Enhancements were delayed until the revised TCC/Voyatek contract is in place. Currently the relationship between enrollment and routine operations are separate.	FRVM staff

	Refinement of the full monthly redemption report with EBT data through CT-WIC. Report captures data from multiple modules and redemption tables.	Data had only been accessible through EPPIC and/or IT report requests.	EPPIC access is not available to the local agency Coordinators, and data must be available as needed.	FRVM staff Nutrition staff IT staff
	Access CT-WIC screens via SIM card technology on a tablet to allow real-time access to vendor information and provide electronic record of monitoring visits.	Monitoring visits currently done on paper.	Number of electronic documents available through the vendor portal to the MIS. Apple iPads can be used to access CT-WIC for in-store use.	FRVM staff
	Explore functionality for Adobe to integrate with Microsoft Teams and utilize for signing of training certificates and/or monitoring visits.	May 2024	Microsoft Office Hours and expanded use of Microsoft products	FRVM staff
4.4 Training videos to supplement interactive vendor training	Recordings through Microsoft Teams to develop training videos to incorporate into open enrollment and renewal interactive trainings.	March 2020	Consistent messaging for all training sessions. All trainings are now on Teams since the Covid pandemic.	FRVM staff
	Post on website for vendor use in training additional staff.	March 2020	Assistance to vendors to train more employees on WIC policies and procedures.	FRVM staff
	Review information gathered from other states who utilize post-training tests, online assessments to test knowledge of training.	March 2020	Training chapters will apply to different areas regarding minimum stocking requirements, transaction processing, violations, and sanctions.	FRVM staff
4.5 Improve compliance investigation services.	Explore the use of summer interns and those seeking non-DPH internships for students to conduct compliance investigations.	January 2024	Collaboration with criminal justice students or Connecticut Department of Emergency Services and Public Protection	FRVM staff
	Explore the use of local agency WIC staff or non-WIC DPH staff to conduct compliance investigations.	January 2024	Issued an RFP for compliance buys investigation. No responses received in FY21.	FRVM staff

	Explore the use of investigators utilized by DCP Liquor Control, State Police or DRS Tobacco Control to conduct compliance investigations.	January 2024	DPH staff have been conducting undercover buys and inventory audits for investigations.	FRVM staff
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Program Functional Area 5: Management Information Systems

Goal 5: To maintain and enhance the WIC IT infrastructure through 9/30/2025.

- Objective: 5.1 Implement solutions or new technologies to modernize participant experience and/or address state and federal requirements.**
- Objective: 5.2 Move toward a self-service reporting environment for regular WIC information needs.**
- Objective: 5.3 Develop a new MIS equipment obsolescence plan.**
- Objective: 5.4 Increase staff knowledge and utilization of current IT languages, tools and techniques.**
- Objective: 5.5 Implement new technologies to enhance productivity or system security.**
- Objective: 5.6 Implement next EBT contract and convert.**
- Objective: 5.7 Implement next MIS support contract.**
- Objective: 5.8 Implement Participant Portal.**

Objective	Strategies/Activities	Baseline	Indicators	Staff Assigned
5.1 Implement solutions or new technologies to modernize participant experience and/or address state and federal requirements.	Add additional functionality to CT-WIC.	CT-WIC implementation project complete.	Timely update of changes to address needs or requirements.	WIC IT Staff
5.2 Move toward a self-service reporting environment for regular WIC information needs.	Implementing data dashboard to replace file distribution.	Webfocus reports have been converted to SQL Server Reporting Services (SSRS)	Begin discussions with agency data team on Power BI or other self-service option; provide additional access to local agency management for DPH Reporting Portal.	WIC IT Staff WIC Epidemiologist Nutrition Consultant
5.3 Develop a new MIS equipment obsolescence plan.	Continuing equipment refresh every three to four years.	Current IT infrastructure is a client/server environment on windows platform using technical control is as follows: Routers are managed by DAS Bureau of Information Technology Solutions, while switches, servers, desktops, laptops, tablets and printers are	Age and maintenance.	WIC IT Staff

		managed by DPH/WIC IT, providing the necessary security, constant monitoring of database and network, equipment refreshment and maintenance.		
5.4 Increase staff knowledge and utilization of current Programming languages, tools and techniques.	Staff have access to training library Training on new version of Focus reports to leverage legacy reports with new dashboard technology.	Staff have access to library as needed.	Staff are utilizing training library.	WIC IT Staff
5.5 Implement new technologies to enhance productivity and system security.	Setup Devops for IT Staff task monitoring and updates.	WIC IT Staff to have access to WIC Board to update and review tasks and tickets all in one place.	IT Staff be able to productively look at open items and backlogs.	WIC IT Staff
5.6 Implement next EBT contract and convert.	Complete state contract for EBT services. Convert from existing vendor to new vendor.	Currently in contract negotiations with FIS.	Contract and conversion completed.	State WIC Director WIC IT Staff FRVM staff Nutrition Unit
5.7 Implement next MIS support contract.	The existing contract expired in 2023. Plan procurement strategy with FNS and start IAPD process.	Waiting meets determination from USDA.	IAPD created and statement of work published.	State WIC Director WIC IT Staff
5.8 Implement participant portal.	Add self service capabilities so participants can pre-enroll in WIC, upload documents, and make appointments.	Environment is ready pending contract to do more work.	Self-service portal development and implementation started.	State WIC Director WIC IT Staff Nutrition Unit

Program Functional Area 6: Caseload Management/Outreach

Goal 6: Effectively reach all eligible individuals as resources allow and achieve the maximum caseload capacity to serve the greatest number of women, infants, and children.

By September 30, 2025:

Objective 6.1: Target a 1% increase over 2024 first trimester enrollment rates. (12.8%-2023)

Objective	Strategies/Activities	Data	Indicators	Staff Assigned
6.1 Target 1% increase over 2024 first trimester enrollment rates.	The State agency developed strategies to improve first trimester enrollment- outreach to Planned Parenthood Southern New England (PPSNE) or local OB/GYN will continue for FY 2025.	<u>2025 WIC Objective:</u> ≥ 40% <u>FFY 2019 Target:</u> ≥ 40.0% Average: 19.9% Range: 3.4% - 33.8% <u>FFY 2020 Target:</u> ≥ 40.0% Average: 17.4% Range: 4.3% - 38.6% <u>FFY 2021 Target:</u> ≥ 40.0% Average: 13.7% Range: 0.0% - 34.3% <u>2022 Target:</u> ≥ 40.0%* Average: 14.0% Range: 0.0% - 42.1% <u>2023 Target:</u> ≥ 40.0%* Average: 12.8% Range: 0.0% - 29.0% 2024 data pending. <u>Data Sources:</u> FFY2019-2020: CT-WIC MIS (Management Information System). *Starting with 2022 State Plan submission, we will report available complete calendar year data based on previous calendar year.	CT-WIC Process Objective Report (FY 2023) 1 st trimester enrollment of pregnant women is greater than or equal to 40%.	Program Monitor Epidemiologist Outreach Team

Program Functional Area 7: Coordination of Services

Goal 7: Strengthen coordination of information and resources with other Connecticut State Agencies to respond to the needs of the WIC clients.

By September 30, 2025:

Objective: 7.1 Maintain active coordination with at least 80% of identified key partners in 2025.

Objective	Strategies/Activities	Indicators	Staff Assigned
7.1 Maintain coordination with at least 80% of identified key partners.	Continue to actively participate in State level MCH Task Forces, Head Start, Oral Health, Lead Prevention and IZ programs, HUSKY (Medicaid Managed Care), Food Policy Council (FPC), DSS and OEC. Continue to coordinate with DPH Food Protection Program, Consumer Protection Agency, and DPH Environmental Epidemiology. Facilitate emerging collaborations and/or executed MOUs with the following: • DPH led-Maternal Mortality and Morbidity Review Committee (MMRC) interagency workgroup. • Pregnancy Risk Assessment and Monitoring System (PRAMS) collaboration <i>Internal MOU executed.</i> • CT Dental Health Partnership (DHP)-<i>MOU draft in progress.</i> • Substance Exposed Pregnancy Initiative or SEPI-CT and DMHAS and DCF Policy Academy <i>Collaboration ongoing.</i> • Office of Early Childhood and Home Visiting <i>Collaboration ongoing.</i>	Improvement of service delivery to mutual clients. Ensure safety of food/formula provided to WIC participants. Current recommendations or guidelines relevant to WIC participants are sent to local WIC agencies.	Nutrition Unit Nutrition and Program Monitors Breastfeeding Unit

Objective	Strategies/Activities	Indicators	Staff Assigned
	- CT Coalition Against Domestic Violence <i>Collaboration ongoing.</i> - MCH Block Grant <i>Provide data and narrative for annual reports.</i> Continue to support WIC & Head Start Better Together Collaboration at 9 local agencies in FY25 via NSA funds and existing contracts. - Sustaining State level activities include Liaison visits, bi-annual survey, and inclusion of Better Together objectives in LAP Outreach Plans. - Sustaining local level activities include co-location, collaborative nutrition and outreach, monthly meetings and As resources allow, continue SNAP-Ed/WIC Program Collaboration to complement WIC nutrition education efforts. Coordinate with SNAP-Ed as needed with local agency workshops that include taste testing and cooking demonstrations at selected sites. Incorporate routine use of SNAP-Ed recipes in WIC education/resources used. Update post-tests to Microsoft Forms.	SNAP-Ed evaluations and feedback from SNAP-Ed/local agencies/students SNAP-Ed Recipes utilized at local WIC agencies. Ongoing implementation of WIC PMAD Screening Protocol.	

Program Functional Area 8: Civil Rights

Goal 8: Establish and administer policies and procedures that ensure client-oriented customer friendly service environment for all applicants and participants and comply with Racial/Ethnic Federal mandate to ensure uniformity and comparability in the collection and use of data as required by OMB standards.

By September 30, 2025:

Objective: 8.1 Verify 100% of local agencies are comply with use of revised nondiscrimination statement requirements and OMB racial/ethnic data collection standards.

Objective: 8.2 Conduct annual civil rights training for state and local agency staff.

Objective	Strategies/Activities	Indicators	Staff Assigned
8.1 Verify 100% of local agencies comply with use of non-discrimination statement requirements and OMB racial/ethnic data collection standards.	During monitoring, request copies of LA developed brochures, handbooks, and/or other publications, webpages or social media accounts and review for proper usage of the nondiscrimination statement. Monitor to verify that Racial/Ethnic Data Collection procedures are followed at local agencies during FY25 reviews.	Each brochure and handout and webpage will contain the current USDA non-discrimination statement. Regulatory compliance as evidenced in monitoring reports.	Monitoring staff
8.2 Conduct annual civil rights training for local agency staff.	Annually, update and train all State and local staff on revised nondiscrimination complaint procedures and forms.	Annual interactive Civil Rights applied session is offered annually.	Program Monitor

Program Functional Area 9: Certification & Eligibility

Goal 9: Improve and maintain program integrity in the areas of certification and eligibility.

By September 30, 2025:

Objective: 9.1 Investigate during MER (Nutrition Monitor) and State audits (Nutrition Consultant Formula Lead (NCFL)), local agency compliance with State WIC Special Formula and Eligible Nutritional policies.

Objective: 9.2 Monitor local agency implementation of updated risk criteria.

Objective	Strategies/Activities	Indicators	Staff Assigned
9.1 Investigate during MER (Nutrition Monitor) and State audits (NCFL), local agency compliance with State WIC Special Formula and Eligible Nutritionals' policies.	For local agencies with MER scheduled, the Nutrition Monitor will use the CT-WIC Program Special Formula Review Form to determine if 80% of agencies reviewed comply with special formula procedures. ▪ Select 5 (per permanent site) participant records with WIC Medical Documentation forms to determine frequency of insufficient "medical rationale" and if process was followed for HUSKY issuance of special formula. ▪ Based on monitoring results, determine areas for improvement in staff training and clarify WIC formula policies and procedures and provide technical assistance as needed. The NCFL will use the CT-WIC Program Special Formula Review Form to conduct a minimum of 3 chart audits per local agency with an Off-Year Self Evaluation. Approximately 20 charts per year will be evaluated by the NCFL. Based on results, local agency liaisons will discuss areas for improvement and address questions or concerns during local agency TA visits. Outstanding formula issues identified will be discussed during	Local agencies will demonstrate proficiency with: Following policy and process in assisting participants in ordering of special formula received through WIC or assisting participants dually enrolled in HUSKY/Medicaid and WIC in obtaining special formula. 100% Medical Documentation Form Up to Date. 70% Medical Documentation Form accurately completed by MD/HCP. 90% Medical Documentation correlates with HUSKY and/or WIC prescription issuance on Family Benefit List (FBL). 80% Appropriate Growth Assessment/evaluation documented.	Nutrition Unit Nutrition Monitor NCFL Epidemiologist Liaisons

Objective	Strategies/Activities	Indicators	Staff Assigned
	training sessions at local agency staff meetings, Program Nutritionists and/or statewide meetings. Provide at least one (1) in-service training to stakeholders about changes to WIC special formula policies (HUSKY primary payor) and procedures by September 30, 2025. Assist local agencies in providing in-service presentations as appropriate.	50% of verbal orders for medical documentation are obtained within 24 hours (WIC only). 25% of Nutritionists follow-up with MD/HCP when indicated. 60% CT-WIC Risk Screen updated based on new information.	
9.2 Monitor implementation of updated Nutrition Risk Criteria.	By the end of first quarter, review if there are new or updated WIC nutrition risk criteria changes, develop training plan, and submit IT request to perform needed modifications to CT-WIC. Provide update to Program Nutritionists by 2nd quarter, on results of post-test for WIC Nutrition Risk Criteria and WIC Nutrition Risk Assessment training. (Self-paced module). Train local agency staff on new or revised criteria at Statewide meetings or using a self-paced module. During monitoring determine if local staff accurately identify and assign new or revised risks. Through State MER observations and chart audits 80% of local agency staff will conduct PMAD screening per protocol and document appropriately.	IT report submitted. Training scheduled for local agencies. Monitoring indicates local agency staff are correctly assigning risks.	Nutrition Unit Monitoring staff Breastfeeding Unit IT Unit

Program Functional Area 10: Monitoring & QA

Goal 10: Conduct on-site programmatic management and nutrition services reviews of local agencies to ensure compliance with Federal and State Regulations.

By September 30, 2025:

Objective: 10.1 Monitor five service regions including satellites.

Objective	Strategies/Activities	Indicators	Staff Assigned
10.1 Monitor five service regions including satellites.	By the end of 1st quarter, develop FY24 monitoring schedule. - Conduct monitoring visits, schedule exit conference within two weeks of completion of fieldwork, and complete exit conference within 30 calendar days. - Prepare program review reports to local agencies within 30 calendar days of the program exit conference. - Respond to local agency CAP within 30 days. Two weeks as best practice. - Synthesize common review findings & responses to CAP in both nutrition services and program operations to update FY24 Goals and Objectives, training, and technical assistance plans. - During routine monitoring, collect data on satellite site operations to determine effectiveness: Location of satellite sites, # of participants served, and Hours of operation	100% of scheduled monitoring visits and reports completed by Sept 2024. 80% of agencies will have their exit conference scheduled within two weeks of completion of onsite visits.	Nutrition Monitor Program Monitor Local agency Liaisons Breastfeeding Unit

Program Functional Area 11: Fiscal Management

Goal 11: Maximize the utilization of WIC NSA and food funds.

By September 30, 2025

Objective: 11.1: Manage the infant formula rebate contract and utilize a new system to ensure accuracy in rebate amounts billed to Abbott Laboratories.

Objective: 11.2: Manage internal systems for accuracy in recording and projecting expenditures to ensure proper 798 Report submission.

Objective: 11.3: Implement enhanced CT-WIC Financial Module to allow local agencies to enter budget and expenditure information to improve timely reimbursements and expenditure review against approved budget.

Objective	Strategies/Activities	Baseline	Indicators	Staff Assigned
11.1 Manage the infant formula rebate contract and utilize a new system to ensure accuracy in rebate amounts billed to Abbott Laboratories.	• Track and monitor monthly utilization of formula. • Utilize the new system to ensure accuracy of rebate amount. • Track billing and receipt of rebate amounts billed to Abbott.	Manual reconciliation.	Enhanced reporting capabilities to assist with tracking changes in pricing and ensuring accuracy of pricing and rebate billed.	State WIC Director Fiscal Unit
11.2 Continue to enhance and manage internal systems for accuracy in recording and projecting expenditures to ensure proper 798 Report submission.	• Analyze WIC approved budget against expenditures monthly. • Maintain automated WIC accounting system. • Coordinate process for personnel charging the WIC Grant.	Budget/expenditure reconciliation.	Enhancements to accounting system will reduce posting inaccuracies.	State WIC Director Fiscal Unit
11.3 Implement enhanced CT-WIC Financial Module to allow local agencies to enter budget and expenditure information to improve timely reimbursements and expenditure review against approved budget.	• Monitor information entered into CT-WIC Finance Module by local agencies. • Ensure that expenditures submitted by local agencies are approved in the budget. • Enhance reporting process for local agencies by utilizing an automated process.	N/A	The system tracks any revisions entered and has enhanced reporting capabilities.	State WIC Director Fiscal Unit

Program Functional Area 12: Data Quality, Analysis & Reporting

Goal 12: Strengthen data outputs and use of data for improved program planning, monitoring, evaluation, and administration.

By September 30, 2025:

Objective 12.1: Improve access to, and the utility and application of, WIC Program data.

Objective 12.2: Increase local agency management understanding and use of data for program planning, monitoring, and evaluation.

Objective	Strategies/Activities	Indicators	Staff Assigned
12.1 Improve access to, and the utility and application of, WIC Program data.	Work with team to streamline data reporting related to: • WIC participation and key demographics • Process and outcome objectives • Benefit issuance & redemption • Authorized vendors Prepare and store data reports in one common location for easy CT WIC staff access.	Data reports that meet CT WIC and USDA needs. All data reports (Monthly, Quarterly, Ad Hoc) are saved on the MS Teams WIC Channel or other central location.	WIC Epidemiologists State WIC Director NSU FRVM IT
12.2 Increase local agency management understanding and use of data for program planning, monitoring, and evaluation.	Prepare and conduct at least two data and reporting webinars for local agency management: • Identify date, time, and location. • Select topics for inclusion, identify training objectives, and prepare slides.	Local agency management training completed. Objectives met.	WIC Epidemiologists State WIC Director NSU